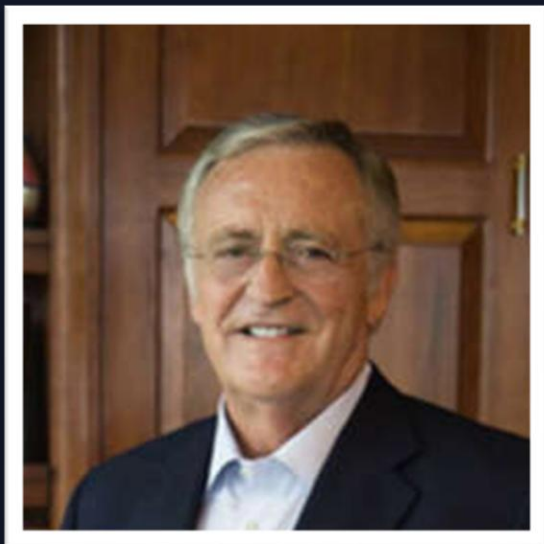


OPPORTUNITY REVIEW

DIRECT-TO- EMPLOYER GTM STRATEGY

Exploring a Med Center Health DTE Health Plan





Our Founding and Heritage: 46 Years of Striving for Honesty in Healthcare

Founded Key Benefit Administrators in 1979

After co-founding a life insurance agency in 1973 and Key Life Insurance in 1976, Larry Dust launched KBA in 1979 as a third-party benefits administrator focused on honest healthcare delivery.

Sold to Blue Cross/Blue Shield in 1985

Strategic partnership with BCBS of Indiana (then part of the Associated Group) provided capital and market access while maintaining operational independence.

Reacquired KBA in 1992

Larry bought the business back from BCBS in 1992, returning it to complete independence and patient-focused mission alignment.

Built the Largest Independent TPA in the U.S.

Under his leadership since the buyback, KBA has become the country's largest independently owned third-party administrator, managing millions of members across diverse markets.

Never Forgotten Who Gives Healthcare

KBA has never participated in reference-based pricing, does not take refunds, and delivers healthcare at 11% below market rates. Our commitment remains with providers and patients, not profit extraction.

Key Benefit Administrators



DISCUSSION OPTIONS

1. Why KBA?
2. Why Med Center Health?
3. Financial Assumptions and Projections
4. Implementation and Operations
5. Network Management

WHY KBA?

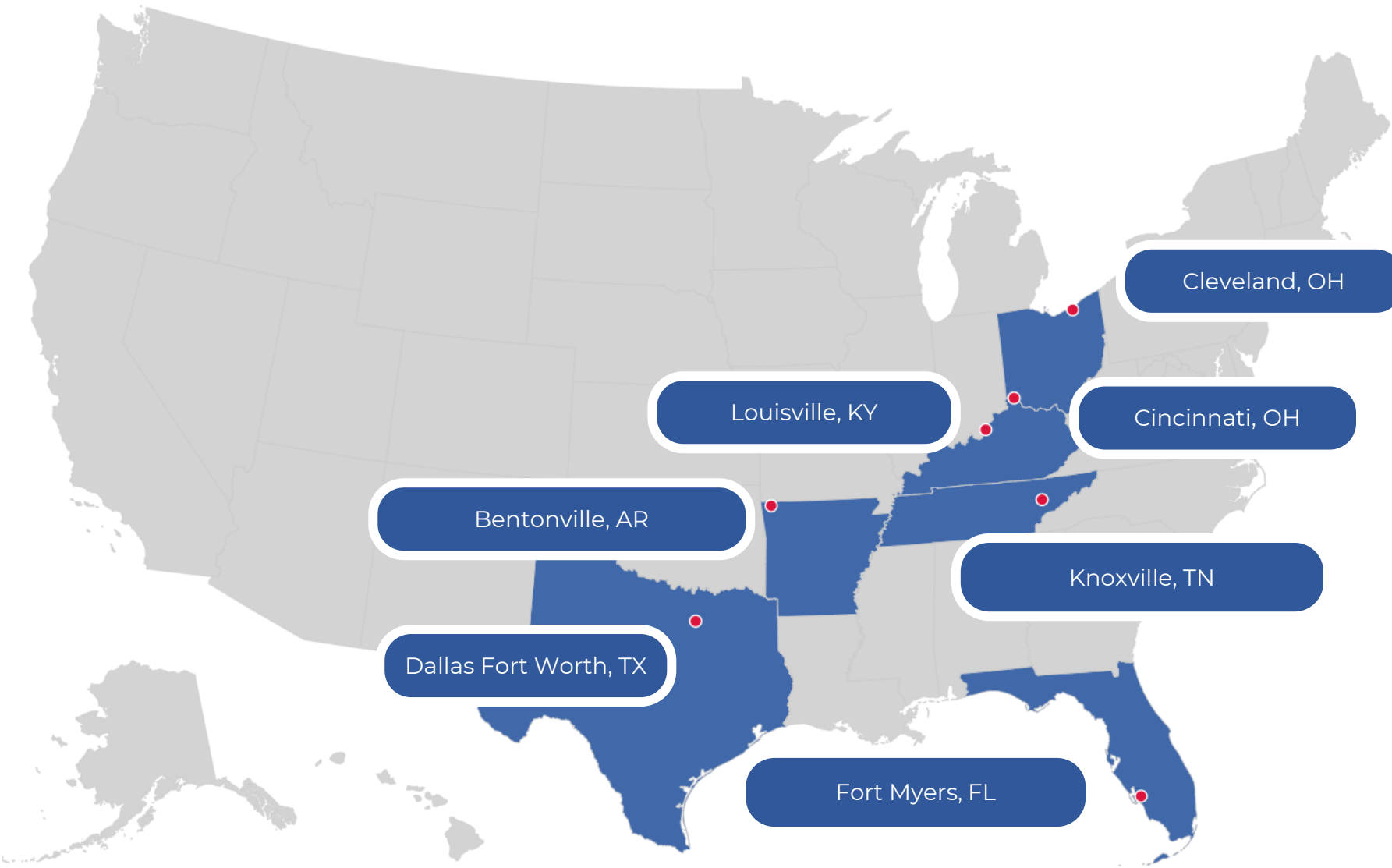




KEY BENEFIT ADMINISTRATORS

- Founded in 1979, headquartered in Indianapolis
- Largest Independently-Owned Full-Service Third-Party Administrator
- Specialize in Administration of Self-Funded Plans and Population Health Management
- Leader in the Direct-to-Employer Movement
- We Know and Understand the Market
- Experience Working with All Professional Teams: Agents, Networks, MGAs
- The Key Family of Companies is a Vertically Integrated group of benefit related organizations
 - 3,000+ Corporate Clients
 - 3 Million Members under Management
 - 700 Employees

MARKET LEADER IN PRIVATE LABELED DTE PLANS



PUTTING A NAME TO THE PROBLEM

CHANNEL DEPENDENCY RISK



PORTER'S FIVE FORCES

Channel dependency risk is a form of bargaining power of suppliers.

EXAMPLES OF CHANNEL DEPENDENCY RISK

Amazon hurts publishing houses

App stores hurt mobile apps

Gig economy platforms hurt gig workers

Netflix hurts film studios

DIRECT-TO-EMPLOYER (DTE)

Your Health Plan
Focused on a Direct
Relationship with Self-
Funded Employers in
Your Market



YOU Pick the Name of the Health Plan



YOU Determine the Fees Charged



YOU Practice the Medicine



YOU Receive the Profits



WE Make it Successful



Direct-to-Employer Strategy: POWERFUL EMPLOYER VALUE

- Reduced medical spend through lower unit costs and better integrated care
- “At the table” with Med Center Health
- Aligned financial incentives
- Complete transparency for unlimited visibility into health benefit plan expenses and cost drivers
- Aggressive stop-loss premiums and policy terms
- Increased employee satisfaction
- Improved cash flow & stabilization of future medical cost trend



Our 'Why': The Photo Moment

THE KBA REWARD THROUGH SEEING A DIFFERENCE

Better Care, Lower Costs, Real Partnerships

The ultimate measure of our success is the moment when all stakeholders gather to review real progress - improved population health and dramatically lower healthcare spending. This is where theory becomes reality, and everyone wins.



All Stakeholders at the Table

Employer, Provider, KBA, and Broker/Consultant
Broker/Consultant working in unified
partnership toward common goals



Reviewing Real Progress

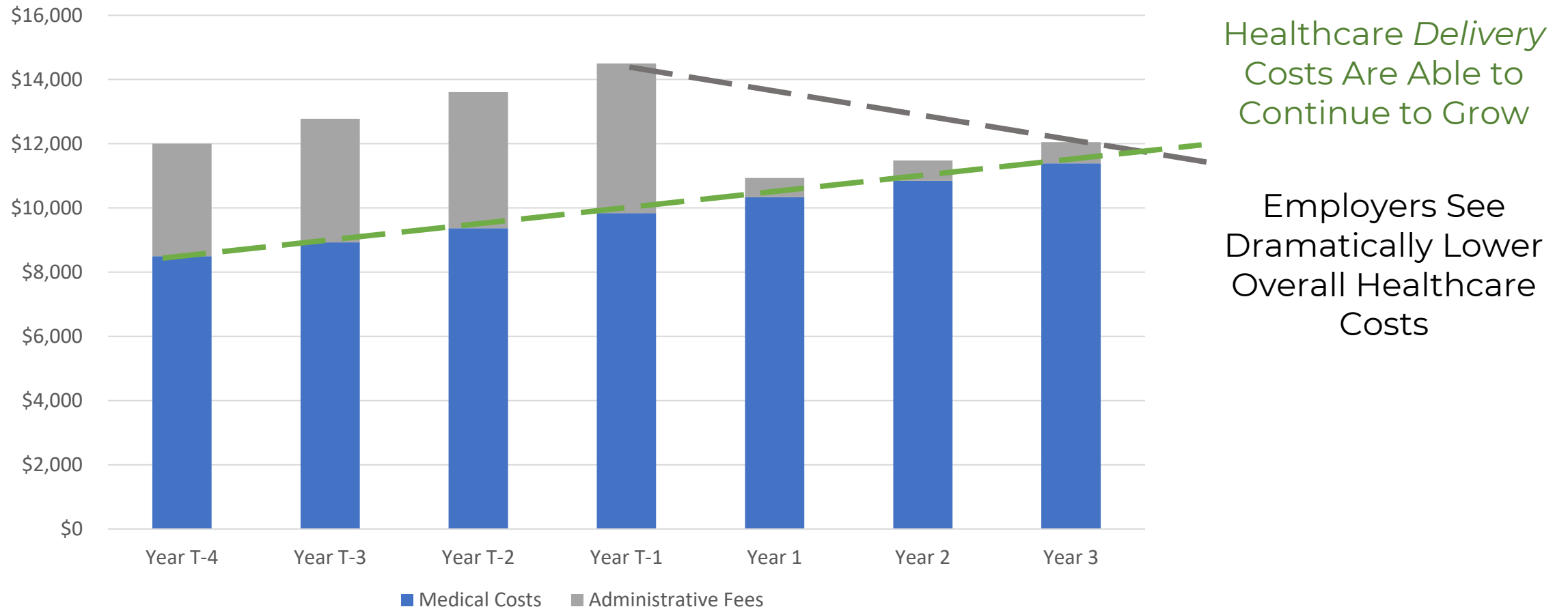
Measurable improvements in population health
health outcomes combined with verifiable
verifiable reductions in total healthcare
spending



Deep Loyalty Built on Value

Authentic relationships between employer and
employer and provider organization based on
based on demonstrated value delivery, not
not contracts

THE WIN/WIN OF CUTTING OUT THE MIDDLEMAN



Proof: 98% of Our Clients Beat Healthcare Inflation

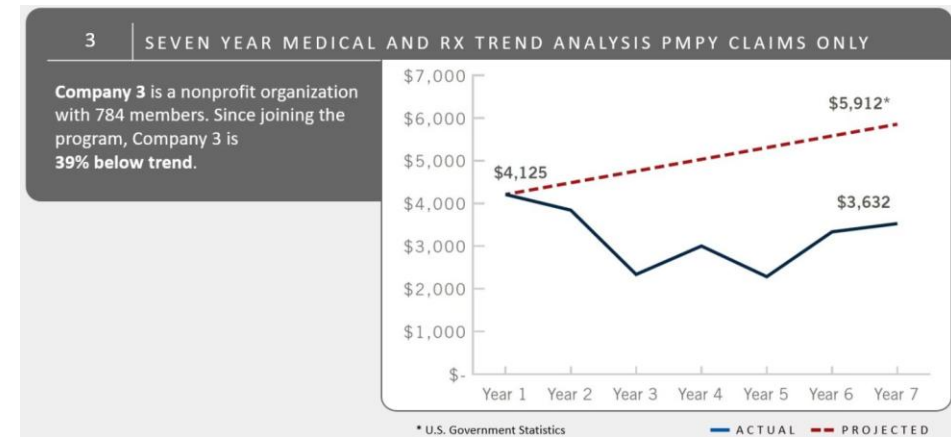
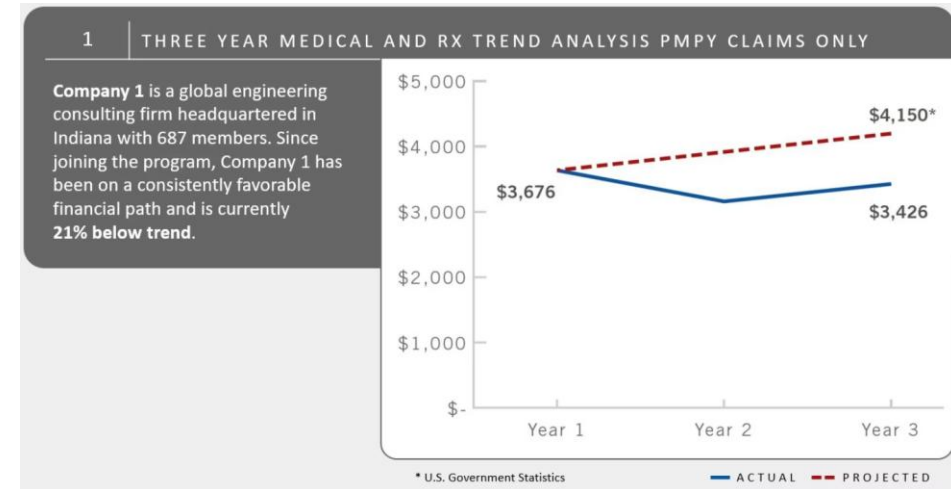
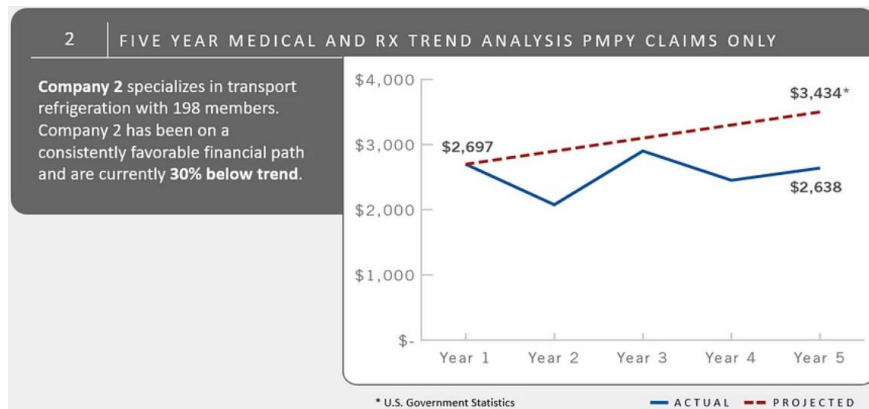
The Red Line = Healthcare CPI

The year-over-year healthcare component of the consumer price index represents typical market healthcare cost increases that employers face nationwide.

The Blue Line = Main Street Management

What happens when you manage your health on Main Street with direct provider relationships, transparent pricing, and aligned incentives.

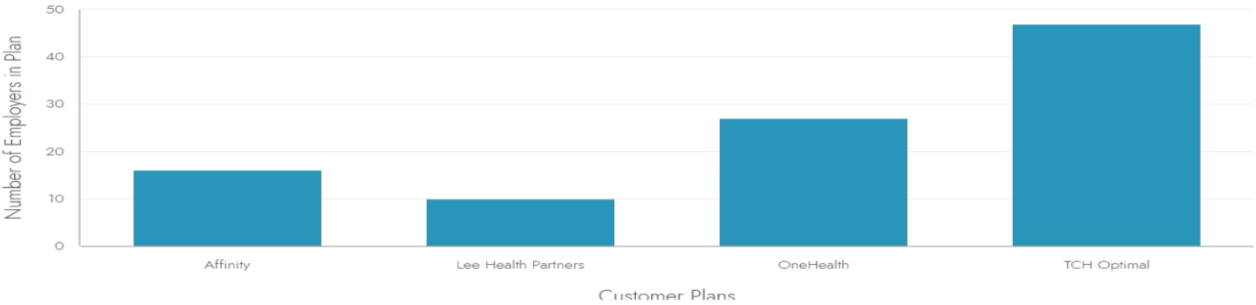
For our clients, 98% of our Blue Lines are BELOW their Red Lines!



This consistent outperformance demonstrates that direct-to-employer partnerships deliver measurable, sustainable value for all stakeholders involved.

KBA Plan Growth in the First Two Years

An average of 25 employer groups in plan within the first two years!



1,600

Louisville KY

OneHealth Plan achieved strong initial growth

1,800

Bentonville AR

Northwest Health Plan exceeded projections

6,400

Cincinnati OH

TCH Optimal demonstrated exceptional market acceptance

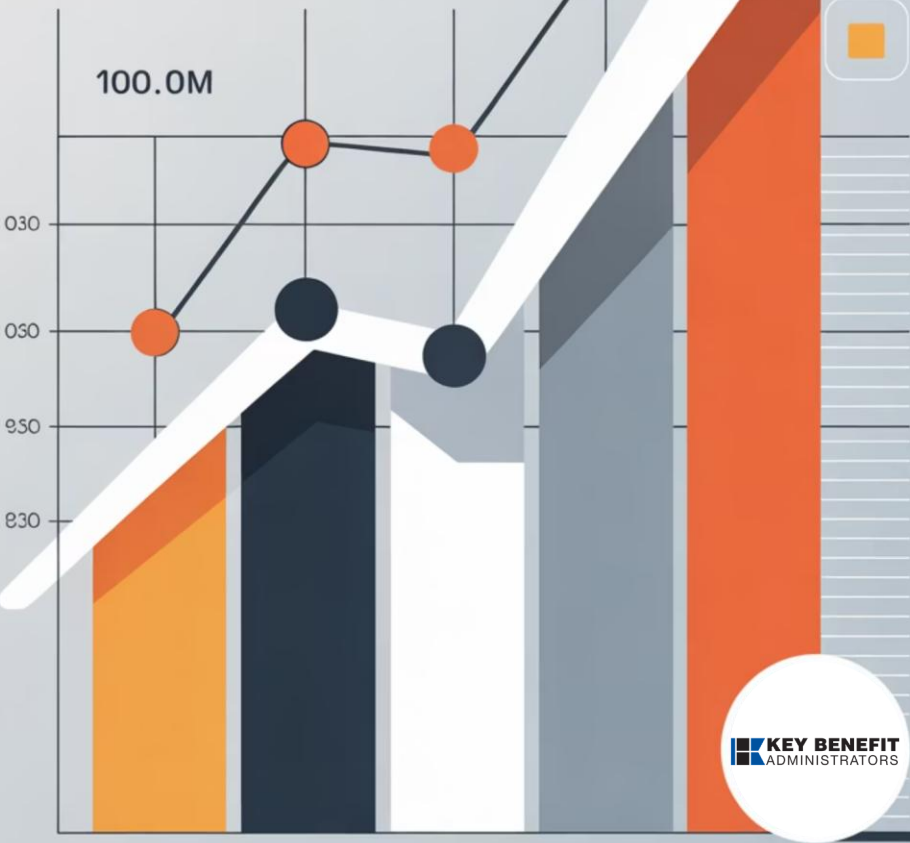
12,900

Fort Myers FL

Lee Health Care Partners achieved remarkable growth

These results demonstrate KBA's proven ability to build sustainable membership growth across diverse market conditions and geographic conditions and geographic regions. Each partnership reflects our commitment to long-term success rather than short-term enrollment term enrollment spikes.

Membership Growth



DTE PROGRAM MARKETING & SALES

KBA WORKS WITH YOU TO MANAGE & SELL:

- Branding & Messaging
- Direct & Brokerage Sales Distribution – Focused Sales Representative
- Brochures, Web Site, Media, etc.
- Stop-Loss/Level Funded Down to 20 EE Lives
- Sell Through Established Brokers in Your Service Area

COMMUNITY BASED SALES STRATEGY



Locally based sales rep with strong community ties



Collaboration with DTE provider partners to identify strategic employers for early adoption and sales



Work with select broker distribution partners with aligned incentives and goals



Pipeline reporting and collaboration with our DTE provider partners



Marketing and promotion of the new option in the community

COMPLETE MARKETING SYSTEM

- Customized Branding and Messaging
- Customized Brochures
- Customized Web Site
- Targeted Employer Outreach
- Sales System (Social Media, Email, Text, etc.)



>>

GTM Frontman Leads Launch
Dedicated rep identified, hired & trained

+

Dual Outreach: Employers + Brokers
Engaged brokers receive outreach acceleration

5x

Target: 5 Engaged Brokers by Month 6
4,500 outreach efforts | 25/day

Month 1 | Market Mapping & Foundation

- Map entire market: all employers, brokers, competitive intelligence
- Identify 10-12 target brokers via Med Center Health recs, national relationships (Aon, NFP, Gallagher) & competitive intel on top producers
- Leverage KBA’s Kentucky market expertise to prioritize targets
- Engage recruiter to hire 1 dedicated local salesperson
- Coordinate with provider marketing & PR teams | 25 outreaches/day begins

GTM frontman leads all activity while dedicated rep is identified & hired

750 cumulative outreaches (5% of 9,000)

Month 2 | Broker Engagement Begins

- Begin broker meetings: present health plan value proposition to all 10-12 target brokers
- Soft-touch employer leader networking underway (events, LinkedIn, referrals)
- Interview process for 1 dedicated local salesperson
- Begin to outline employer target list & joint marketing / PR plan
- Engage PR firm | Establish baseline broker production reporting framework

1,500 cumulative outreaches (17% of 9,000)

Month 3 | Rep Onboarded, Cadence Set

- Hire 1 local salesperson; enroll in KBA training program (4 weeks)
- Dedicated rep shadows GTM frontman; joins broker meetings
- Continue dual outreach: broker engagement + soft-touch employer networking
- Refine employer target list based on broker feedback
- Construct digital sales campaign cadence for broker and employer channels

2,250 cumulative outreaches (25% of 9,000)

Month 4 | Dual Track Outreach Accelerates

- Dedicated rep fully active in local market
- Track deeply engaged brokers -- begin offering outreach support services to accelerate their market engagement and build loyalty
- For engaged brokers: do NOT contact their employer clients directly
- Continue soft-touch employer networking across all channels daily
- Begin joint marketing plan execution (provider + health plan)

Engaged brokers identified -- outreach acceleration services begin

3,000 cumulative outreaches (33% of 9,000)

Month 5 | Broker Loyalty & Employer Coverage

- Deepen engaged broker relationships; expand outreach acceleration services
- Soft-touch employer networking continues daily across all channels
- Distribute seminar invitations; hold DTE seminars
- Lead follow-up from seminars and digital sales campaign
- Q1 broker production report card -- full pipeline review with leadership

3,750 cumulative outreaches (42% of 9,000)

Month 6 | Milestone: 5 Engaged Brokers

- Reach target of 5 deeply engaged brokers by end of Month 6
- Finalize list of non-engaged brokers -- prepare aggressive employer outreach plan for Month 7+
- Engaged brokers continue receiving full outreach acceleration services
- Execute PR plan; lead follow-up from seminars
- Set pipeline review cadence with provider leadership for Phase 2

Month 6: Assess broker engagement status & activate Phase 2 plan

4,500 cumulative outreaches (50% of 9,000 -- Halfway!)

!

Employer Outreach Launches Month 7
Aggressive targeting of non-engaged broker accounts

Month 7 | Phase 2 Launch

- Begin aggressive outreach to employers in non-engaged broker territories
- Continue full outreach acceleration support for all engaged brokers
- Maintain soft-touch employer networking at leadership level daily
- Launch second-tier digital sales campaign
- Begin agency (broker) value proposition training

Aggressive employer outreach launches for non-engaged broker accounts

5,250 cumulative outreaches (58% of 9,000)

Month 10 | Deepening Coverage

- Continue aggressive employer outreach in non-engaged broker territories
- Expand employer leader networking -- target senior decision-makers
- Assess broker engagement: determine if any non-engaged brokers warrant re-engagement
- Prepare annual strategy materials and broker production report card data

7,500 cumulative outreaches (83% of 9,000)

OK

Engaged Brokers: Fully Supported
No direct employer contact in their accounts

Month 8 | Multi-Track Execution

- Continue aggressive employer outreach in non-engaged broker territories
- Engaged brokers: deepen partnership; monitor their employer pipeline activity
- Soft-touch employer networking at leadership level ongoing daily
- Co-marketing initiatives with engaged brokers underway
- Q2 broker production report card issued to leadership

6,000 cumulative outreaches (67% of 9,000)

Month 11 | Final Push

- Final push on employer outreach in non-engaged broker markets
- Engaged brokers: annual performance review preparation
- Soft-touch employer networking at leadership level -- reinforce relationships
- Set date, time, and cadence for Year 2 pipeline reviews with provider leadership
- Consolidate all outreach data for 12-month review

8,250 cumulative outreaches (92% of 9,000)

#

9,000 Total Outreaches by Month 12
Quarterly report cards + 12-month pivot review

Month 9 | Pipeline Review & Adjustments

- Mid-year pipeline review: assess which non-engaged brokers may be convertible
- Evaluate employer outreach conversion rate in non-engaged territories
- Engaged brokers: quarterly check-in on production and pipeline health
- Continue all daily outreach (25/day target maintained)
- Adjust messaging and targeting based on conversion data

6,750 cumulative outreaches (75% of 9,000)

Month 12 | 360-Day Milestone & Pivot Review

- 9,000 total outreach efforts achieved (25/day x 360 days)
- Annual broker production report card: full pipeline assessment with leadership
- Evaluate engaged vs. non-engaged broker outcomes and ROI
- Assess employer direct outreach results in non-engaged territories

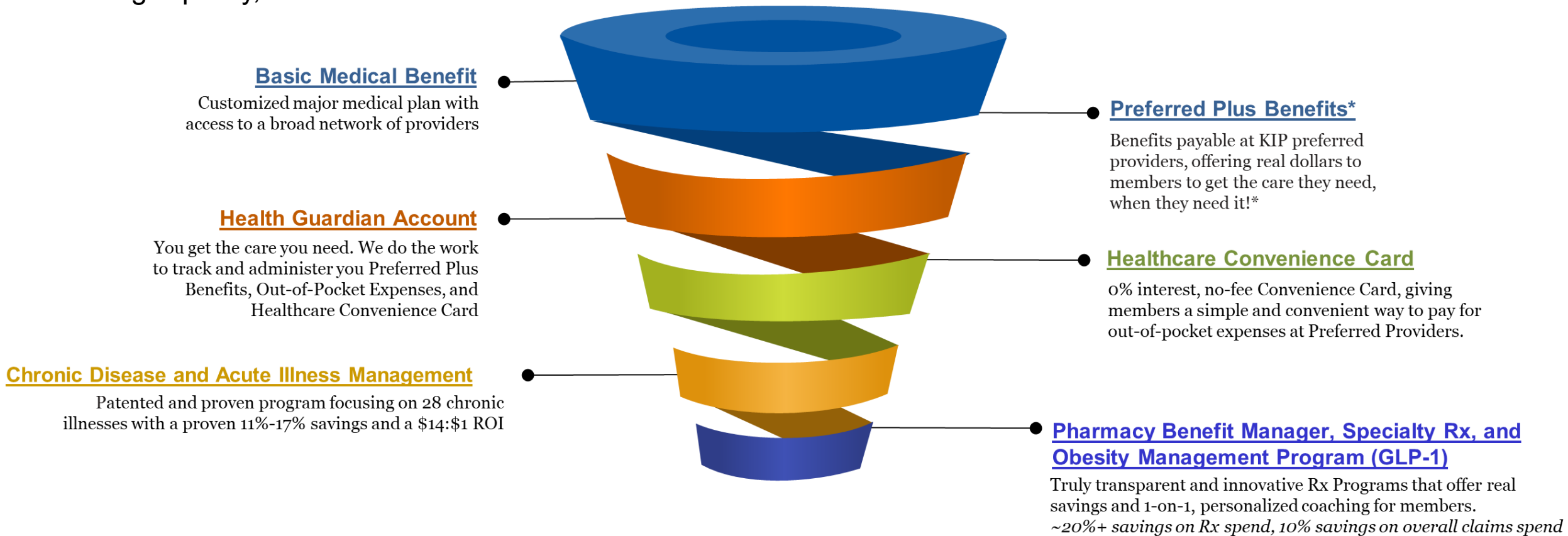
12-Month Check-Up: evaluate potential pivot to expanded broker distribution

9,000 outreaches complete | 100% of target achieved

The Key Incentive Plan

Affordability Without Compromise

The **Key Incentive Plan** is a one-of-a-kind solution, empowering employers to take back control of their healthcare programs, that offers freedom of choice when choosing providers in your local community, while incenting members to choose high-quality, cost-effective care.



Weight Management (GLP-1s) that Don't Break the Bank

The GLP-1 medication market has experienced explosive growth, fundamentally reshaping healthcare spending across America. Prime Therapeutics reports that per-member-per-month spend has tripled by March 2025, with these breakthrough medications now representing approximately 15% of total pharmacy spend. More than 1 in 50 members filed a GLP-1 claim in Q1 2025 alone, demonstrating unprecedented demand for these life-changing treatments.

The Challenge

Traditional GLP-1 medications cost between \$1,089-\$1,349 monthly, creating monthly, creating unsustainable budget pressures for employers while limiting employee access to essential treatments.

Our Solution

Key Benefits secures generic GLP-1 medications for just \$249 per month, enabling comprehensive workforce health while maintaining strict budget control and financial stability.

The Impact

Deliver a healthier population, increased employee satisfaction, and sustainable costs that support long-term organizational success and success and employee wellbeing.

Proven Results: \$7M Annual Savings

One major city spending \$11 million annually on GLP-1 medications achieved \$7 million in cost savings through our comprehensive approach. This dramatic reduction demonstrates how strategic sourcing combined with clinical support creates sustainable, scalable solutions for large employers facing escalating pharmaceutical costs.

Beyond Medication: Comprehensive Care

Our program extends far beyond cost-effective medication access. Members receive specialized coaching twice monthly, focusing on maintaining muscular strength, engaging in moderate exercise, and developing sustainable lifestyle changes that support long-term weight maintenance after GLP-1 treatment completion.

270

Health Conditions

Created or exacerbated by obesity according to Cleveland Clinic research

\$8.5K

Additional Spend

Average annual healthcare costs above normal for obese individuals (KFF)

✔ **Transform Your Healthcare Strategy:** Join forward-thinking employers who are providing comprehensive weight management solutions while protecting their bottom line. Our approach combines clinical excellence with fiscal responsibility, creating a win-win for organizations and their valued employees.

DTE Covered Members Financial Analysis

Target Estimate Assuming 50%+ Within-Plan Steerage and a Commercial Margin of 33.6%.

DTE Members	Pre-DTE Revenue	Pre-DTE Margin	DTE Revenue	DTE Incr. Revenue	DTE Total Margin	DTE Incr. Margin	Net Margin Less DTE Fees
100	\$ 268K	\$ 90K	\$ 532K	\$ 265K	\$ 179K	\$ 89K	\$ -240K
500	\$1,340K	\$ 450K	\$ 2,662K	\$1,323K	\$ 895K	\$ 444K	\$ 120K
1,000	\$ 2,680K	\$ 900K	\$ 5,325K	\$ 2,645K	\$1,789K	\$ 889K	\$ 571K
2,500	\$ 6,699K	\$ 2,251K	\$13,312K	\$ 6,613K	\$ 4,473K	\$ 2,222K	\$ 1,922K
10,000	\$26,796K	\$ 9,003K	\$ 53,246K	\$26,451K	\$17,891K	\$ 8,887K	\$ 8,677K
20,000	\$53,591K	\$18,007K	\$106,492K	\$52,901K	\$35,781K	\$17,775K	\$17,685K

- NOTE: Please see detailed financial analysis and assumptions used starting on page 28.
- Financial projections do not include 340B contribution margin, which could represent \$136 PMPM by optimizing 340B through Rx plan design, Med Center Health's approved site(s) with network integration, and development of Med Center Health's proprietary PBM program.

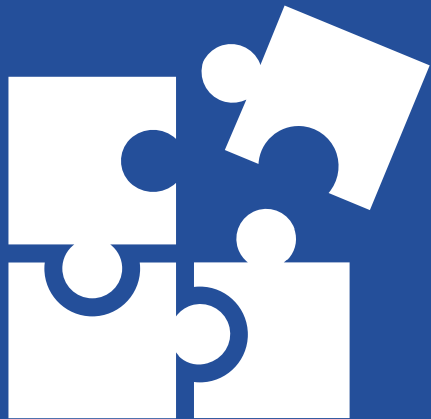
INTEGRATION WITH EPIC EHR SYSTEM

**KBA shares
evidence-based
standards of care for
27 chronic conditions
directly into Epic**



Hyperthyroidism: Yearly Clinical Evaluation, TSH and T4

THE MISSING PIECE TO VALUE- BASED CARE



- Patient engagement is the weak link in delivering value-based care; Med Center Health's clinical leadership and employer participation can drive better outcomes.
- KBA's proactive care-management team supplements your care-management efforts in outreach to clients.
- Med Center Health excels in Standards of Care completion rates for the Chronically Ill, improving care and costs.
- Integrating Med Center Health's managed care with KBA's programs positions Med Center Health as the value leader.

TRANSPARENT PBM INTEGRATION

- Support of Med Center Health's retail pharmacy(s)
- Optimize Med Center Health's 340B drug pricing program
- Allows Med Center Health to create its own PBM program if desired
- Custom telephone number with personalized scripting and separate call tracking & reporting
- Fully customizable member welcome booklet & materials
- White labeled formulary document and marketing materials

CUSTOM CLIENT SUPPORT:

- Specialized call center capabilities
- Fully customizable plan designs
- Operational access to the platform
- Customized process and functions to promote DTE strategy

CLINICAL FLEXIBILITY:

- Full portfolio of custom formulary offerings – with optimized formulary shield
- Clinical programs that drive towards overall DTE strategy
- Options for additional drug savings by driving members to lower cost alternatives
- Flexibility to customize or layer on utilization management programs

STOP-LOSS INSURANCE

THE KBA EXPERIENCE

- Deep relationships with 20+ stop loss underwriters and carriers
- Over \$150M in annual stop loss premium placement
- Comprehensive pricing and medical service analysis performed for each DTE health plan to ensure proper pricing
- KBA provides the administration required of the stop-loss markets:
 - Bordereau Reporting
 - Claim Reporting and Reconciliation
 - Evaluation of Networks
 - Care Management
 - Initial Underwriting
 - Negotiation of rates and terms
 - Plan Design Actuarial

Tokio Marine

Nationwide

Berkshire

Hathaway

Sun Life

Liberty Mutual

Granular (Google)

Companion Life

Crum & Forster

American Fidelity

RGA



36.5% OF AMERICANS DELAYING MEDICAL CARE DUE TO COST

The high cost of healthcare in the United States is a significant barrier to accessing necessary medical services. According to recent data, over a third of Americans have had to skip or delay their medical care in the past year due to the financial burden. This is a concerning trend that highlights the need for more affordable and accessible healthcare options.

The Key Incentive Plan

Base Medical + Preferred Plus Benefits = Your Deductible Can Simply Vanish

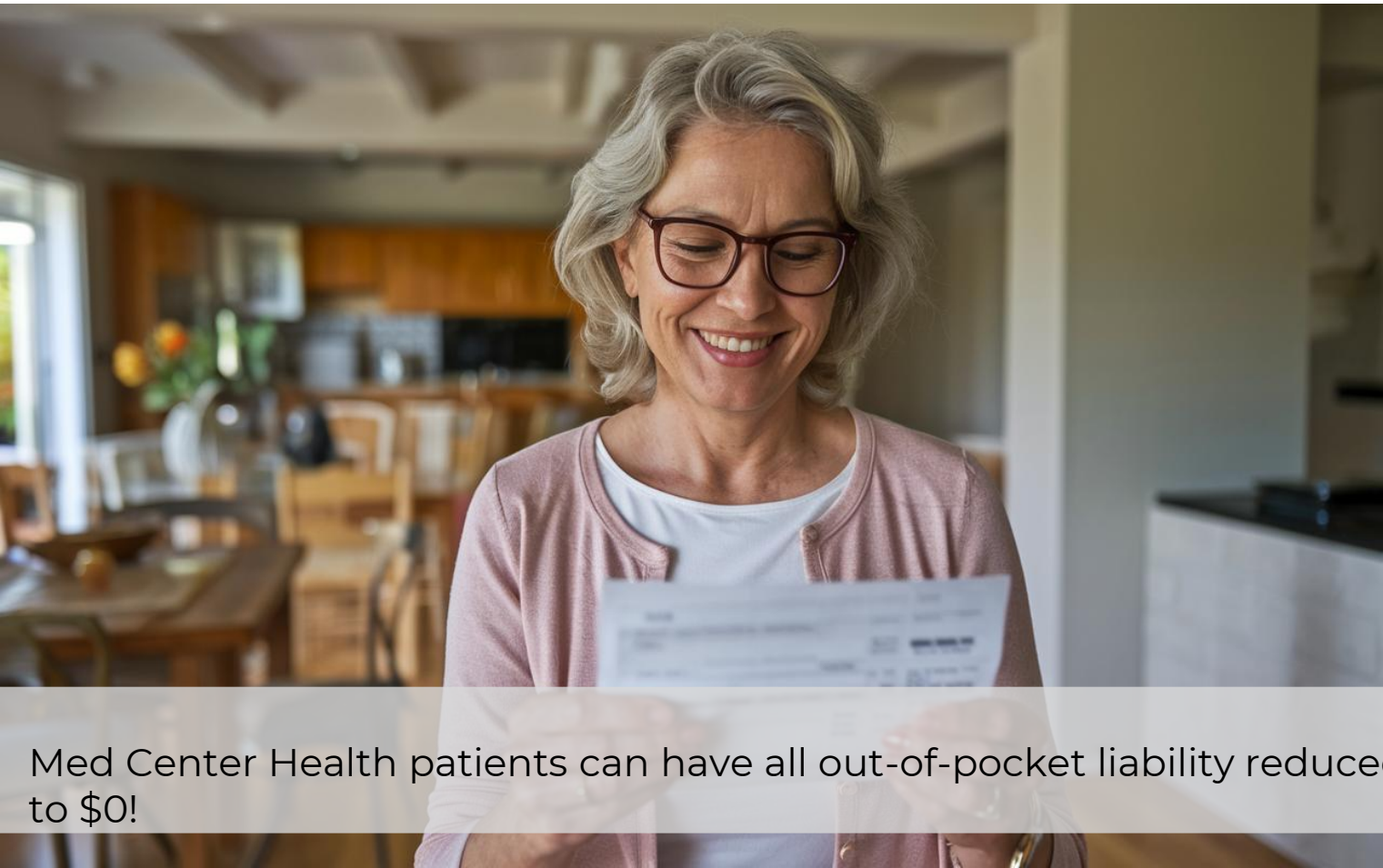
Example: Member with a \$2,500 deductible / \$4,500 max out-of-pocket has a heart attack and cardiac surgery, resulting in a 5-day in-patient stay. In this example, the member would have a \$4,500 OOP with \$5,050 Preferred Plus Benefits for a **net OOP \$0 due to the preferred provider versus \$4,500 OOP exposure at any non-preferred provider.**

BASE MEDICAL BENEFITS*		PREFERRED PLUS BENEFITS		
Deductible		Benefits automatically included in your plan!		
Single	\$2,500		Preferred Provider	Non-Preferred Provider
Family	\$4,500	OP Radiology/X-Ray	\$200	\$0
Coinsurance		OP Diagnostic Labs	\$100	\$0
80% / 20%		Preferred Hospital Stay (\$500/day, 5 days)	\$2,500	\$0
Maximum Out-of-Pocket		Critical Illness Benefit	\$2,000	\$0
Single	\$4,500	Health Screening Benefit	\$125	\$0
Family	\$9,000	Total Employee OOP	\$4,500	\$4,500
Chronic Disease Management Regimens of Care Covered at 100%		Preferred Plus Benefits	\$4,925	\$0
Healthcare Convenience Card to Cover Out-of-Pocket Expenses at 0% interest		Employee Amount Due to Provider	\$0	\$4,500
Health Guardian Account to help members manage their healthcare expenses at Preferred Providers		Additional Benefit Payable to the Health Guardian Account	\$425	\$0

*Base Medical Benefits are for illustration purposes only. Base benefits can be customized to meet client needs.

THE BENEFIT STEERING THE PATIENT TO MED CENTER

What Will this Claim Cost Me?



Med Center Health patients can have all out-of-pocket liability reduced to \$0!

KEY BENEFIT
ADMINISTRATORS

Explanation of Benefits	
Heart Attack, Cardiac Surgery	
THE CHARGES	
Medical Bills	\$25,000
Healthcare out-of-pocket due to provider	\$4,500
Since your out of pocket maximum has been met, the rest of your eligible charges this year will be paid at 100%	
THE CREDITS	
OP Radiology/X-Ray	\$200
OP Diagnostic Labs	\$100
Preferred Hospital Stay (\$500/day, 5 days)	\$2,500
Critical Illness Benefit	\$2,000
Health Screening Benefit	\$125
Benefit Total	\$4,925
Out of Pocket Paid to Hospital	(\$4,500)
Balance Paid to HGA	\$425

THE KBA HEALTHCARE CONVENIENCE CARD

Creating EPO Steerage in a PPO Network

The Healthcare Convenience Card is eligible to be used only for specific merchant codes.

1. To utilize at Specific Healthcare Providers
 - a. Designated Hospitals
 - b. Urgent Care Facilities
 - c. Physicians
 - d. Eligible Ancillary Providers
2. Out of Pocket Expenses Qualify
3. 0% Interest Rate
4. Employee Determines Repayment Schedules
5. To Qualify: Answer One Simple Question



WHY Med Center Health?



**DIRECT-TO-EMPLOYER STRATEGY:
ONLY AVAILABLE TO *PREMIER* HEALTH SYSTEMS**



Med Center
Health

STRONG Brand Quality

EXTENSIVE Patient Access

AFFORDABLE Value

Pricing

EXCELLENT Care Quality

MED CENTER HEALTH IS PATIENT RECOMMENDED

Health System	HCAHPS Recommendation Percentile	CMS Stars Percentile	Mortality Percentile	Readmission Percentile	KBA Brand Quality Score
Med Center Health	47	31	63	72	61
HCA Healthcare	44	46	75	53	60
TJ Regional Health	30	63	67	47	56

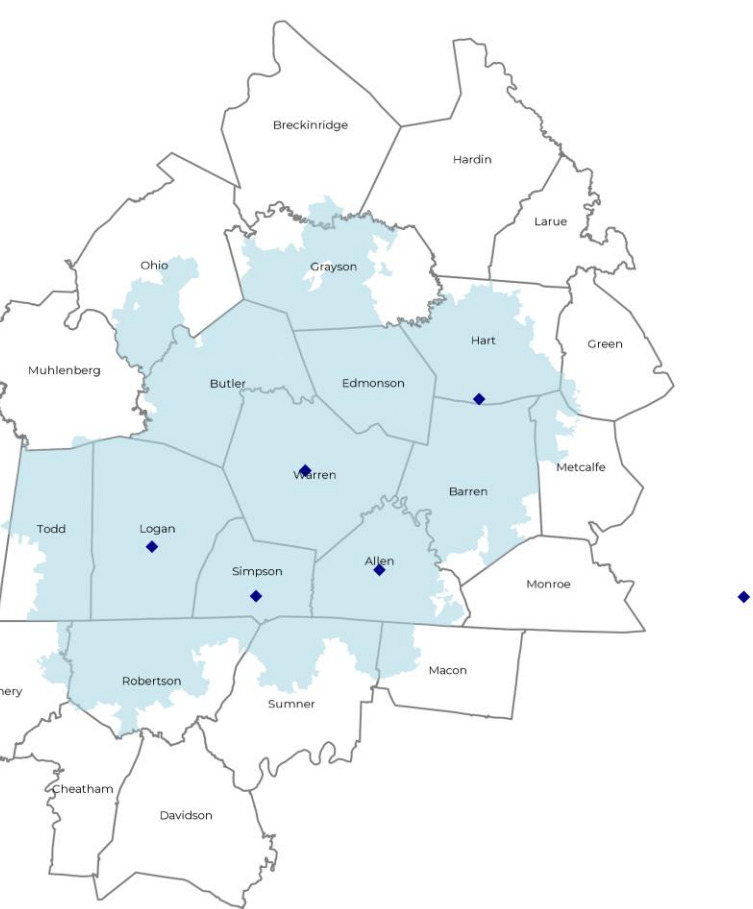
Based on quality measures and HCAHPS feedback, KBA estimates that Med Center Health has the strongest brand quality in Allen, Barren, Butler, Edmonson, Grayson, Hart, Logan, Metcalfe, Ohio, Simpson, Todd, Warren, Montgomery, Robertson, and Sumner Counties.

SIGNIFICANT OPPORTUNITY FOR PLAN GROWTH

Researched for the below highlighted service area

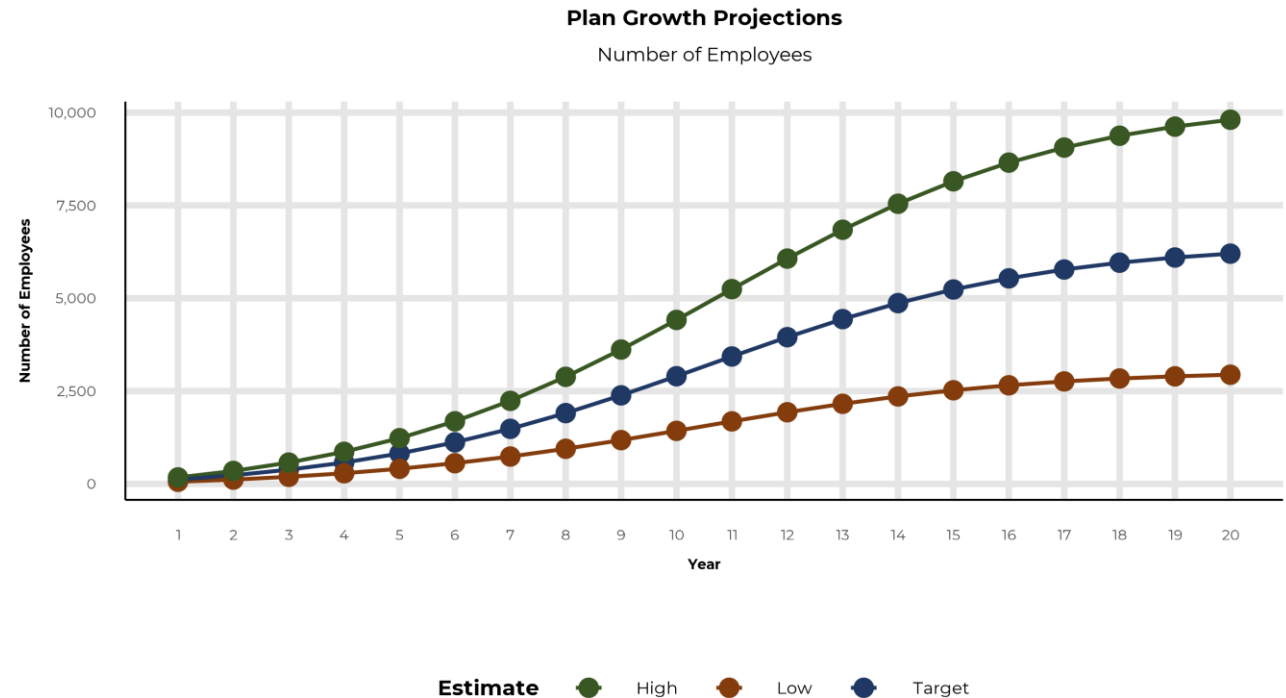
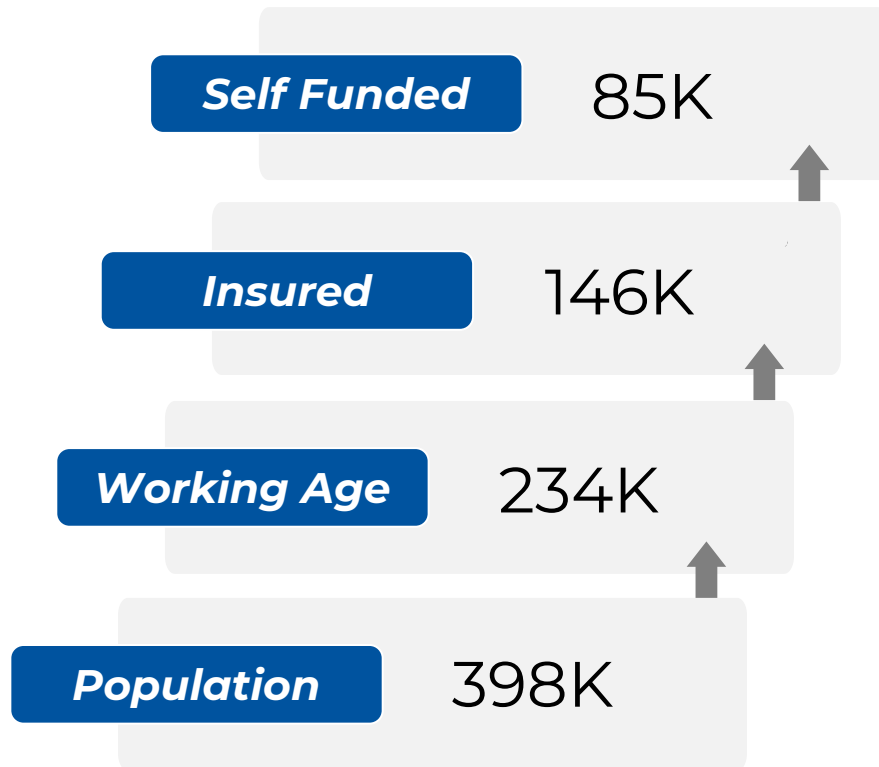
Target Market

250+ Employers ➤ 60,000+ Employees ➤ 120,000+ Members



	Employer Size Group (# of Employees)					
Commercial Employers With Headquarters In Catchment Area	50-100	101-200	201-400	401-1,000	1,001-3,000	3,000+
	134	54	39	26	9	3
Total Employees at Employers Headquartered in Catchment Area	50-100	101-200	201-400	401-1,000	1,001-3,000	3,000+
	10,100	8,100	11,700	18,200	18,000	18,000
Total Members with Employers Headquartered in Catchment Area	50-100	101-200	201-400	401-1,000	1,001-3,000	3,000+
	18,200	14,600	21,100	32,800	32,400	32,400

PLAN POTENTIAL GROWTH PATHS



Estimates includes the Med Center Health service area in Allen, Barren, Butler, Edmonson, Grayson, Hart, Logan, Metcalfe, Ohio, Simpson, Todd, Warren, Montgomery, Robertson, and Sumner Counties

GROWING POWERFUL, LASTING PROFITABILITY

Early Stage

Gaining Name Recognition, Early Growth

New Revenue: **\$1.04M**
New Profit: **\$0.16M**

Growth Stage

Strong Reputation, Strong Growth

New Revenue: **\$5.88M**
New Profit: **\$1.87M**

Mature Stage

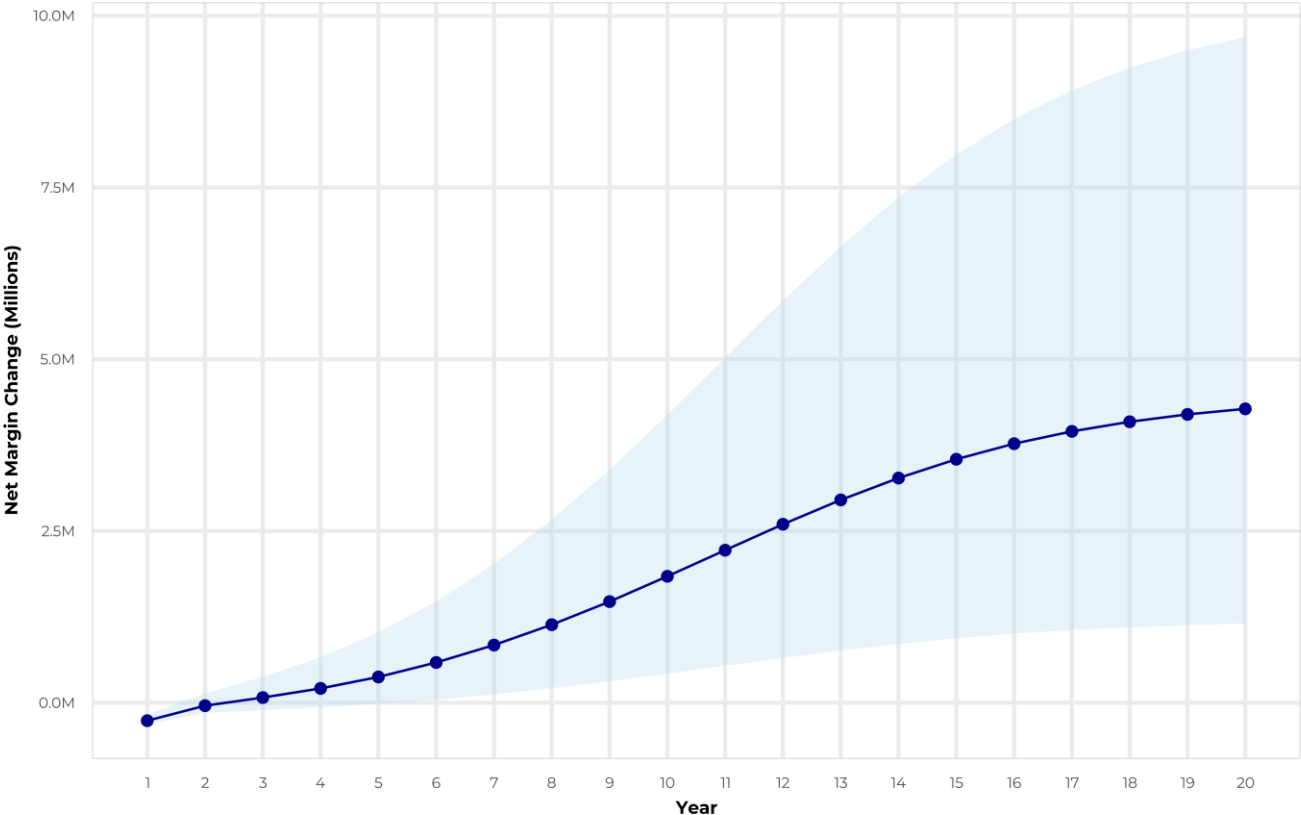
High Profitability, Market Saturation

New Revenue: **\$11.65M**
New Profit: **\$3.87M**

Net Margin Change Over Years

Variation by Member Growth With Target Estimate Assuming

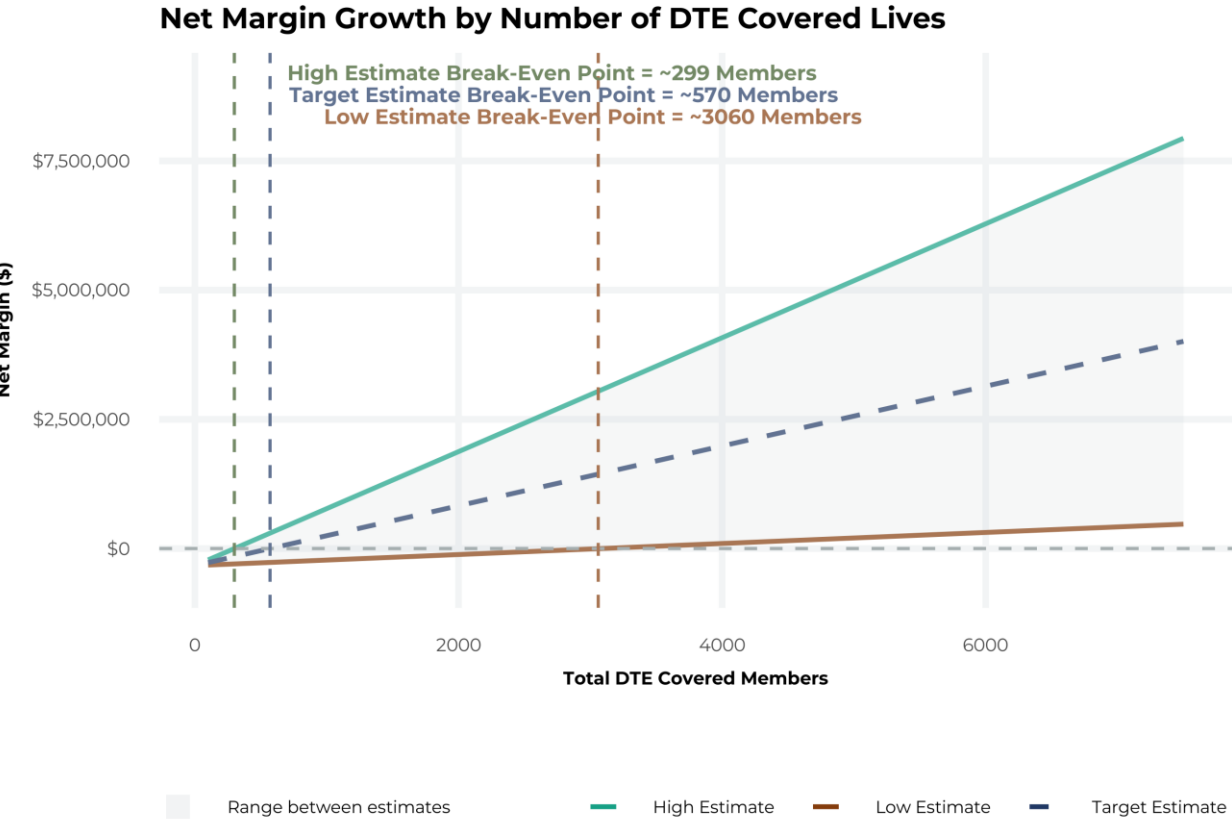
50%+ Within-Plan Steerage and a Commercial Margin of 33.6%



The image is a composite. The upper portion shows a panoramic view of a city skyline, likely New York City, with the Empire State Building as a prominent feature, seen through a window with vertical frame lines. The lower portion shows a close-up of a desk with several sheets of paper containing financial charts and tables. A black pen, a white coffee cup on a saucer, and a calculator are also on the desk. A semi-transparent white banner is placed across the middle of the image, containing the title text.

FINANCIAL PROJECTIONS & ASSUMPTIONS

MODEL ASSUMPTIONS AND BREAK EVEN



Patient Steerage Assumptions

KBA currently estimates that Med Center Health has 70% inpatient market share, and 50% outpatient market compared to hospitals/clinics in the Med Center Health service area (see slide 8). The DTE tiered structure will create immediate steerage, with additional steerage over time from patient bonus benefits and the patient convenience card (see slide 34 for estimates of target steerage).

Low	High
10% below target	10% above target

Commercial Margin Assumptions

KBA observes a Med Center Health 42% commercial margin. Conservatively, we model the DTE to be even with commercial rates given prices 10% below market, but with a 10 percentage-point increase through friendlier billing practices and improved patient collections with the patient card.

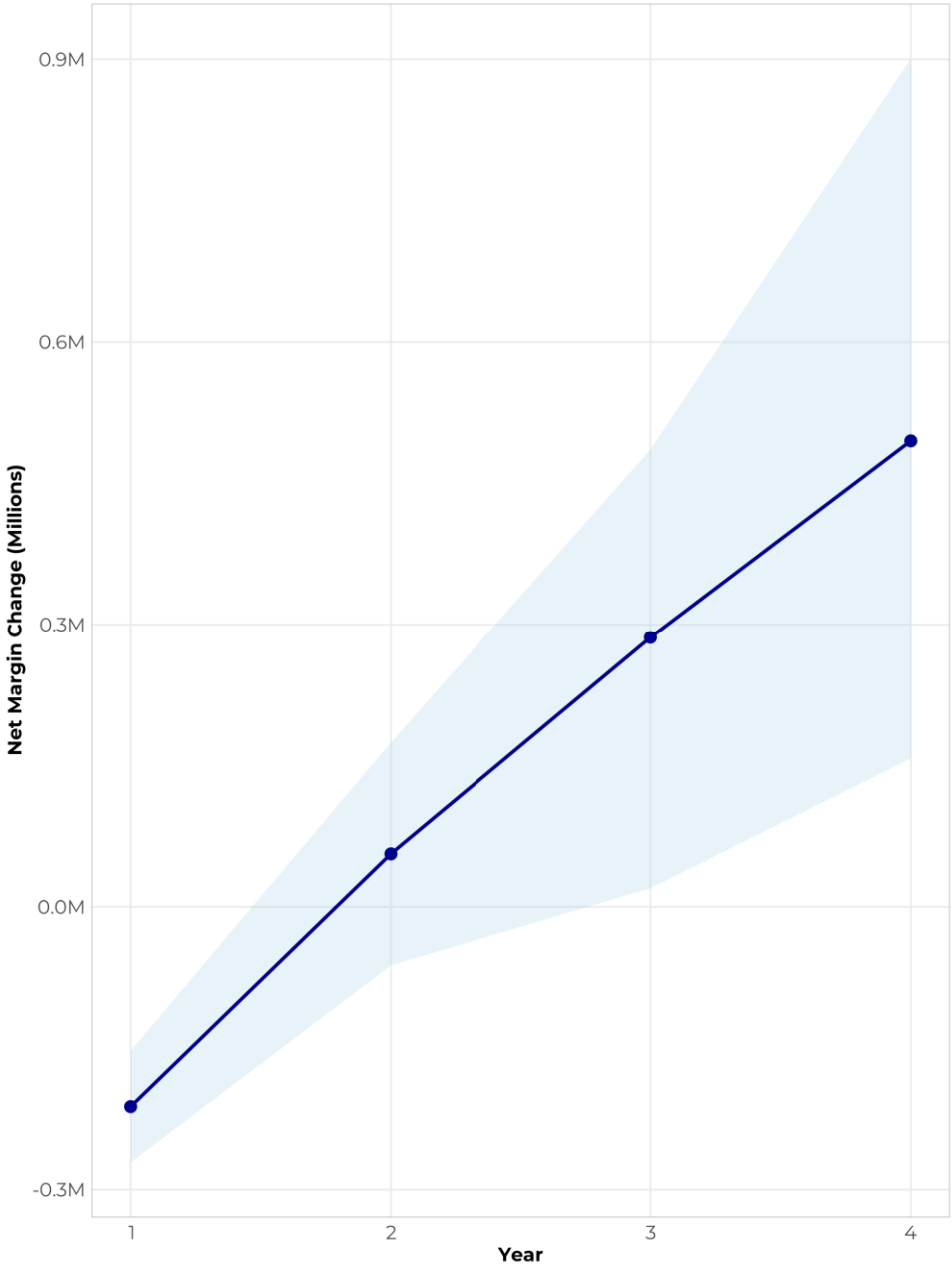
Low	Target	High
31.1%	33.6%	36.1%

FOUR-YEAR PROJECTIONS

Net Margin Change Over Years

Variation by Member Growth With Target Estimate Assuming

50%+ Within-Plan Steerage and a Commercial Margin of 33.6%



Target Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	200	400	700	1,000
Incremental Margin	115.7K	231.4K	457.8K	663.3K
Network Access Fee	2.4K	4.8K	8.4K	12.0K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	-211.9K	56.2K	286.2K	495.3K

High Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	300	600	1,000	1,600
Incremental Margin	173.5K	347.0K	653.9K	1,061.3K
Network Access Fee	3.6K	7.2K	12.0K	19.2K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	-152.9K	174.2K	485.9K	900.5K

Low Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	100	200	300	500
Incremental Margin	57.8K	115.7K	196.2K	331.6K
Network Access Fee	1.2K	2.4K	3.6K	6.0K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	-271.0K	-61.9K	19.8K	157.6K

ESTIMATION OF MED CENTER HEALTH'S 2023 PAYER MIX

Payer Group	Total Inpatient Days	Revenue	Payer Mix
All Med Center Health	102,850	\$667,836K	100%
Medicare	30,233	\$89,297K	28%
Medicaid	3,356	\$175,022K	3%
Other Government Pay	0		
No-Pay, Charity Care	1,477	-\$10,391K	1%
Commercial/Self Pay	67,784	\$403,658K	68%

Values in blue are taken from the 2022 Medicare Provider Cost Report (<https://data.cms.gov/provider-compliance/cost-report/hospital-provider-cost-report/data>)

Values in yellow are taken from Definitive Healthcare data.

Total Medicare patient revenue is reported by CMS in the Medicare Provider Analysis and Review Report (MedPAR)

Estimating Payer Mix

- Medicare, Medicaid, and other government payer mixes are estimated as the percentage of inpatient days for each group divided by total inpatient days.
- No-pay and charity care is estimated as the percentage of no-pay/charity care compared to total overall revenue.
- Commercial/self pay is assumed to be the rest of the total payer mix share.

Estimating Total Inpatient Days

- Total inpatient days for 'No Pay' and 'Commercial' are estimated to be the payer mix estimates for "No Pay, Charity Care" and "Commercial/Self Pay" multiplied by the total number of inpatient days.

In other analysis, the outpatient payer mix is assumed to mirror the inpatient payer mix.

ESTIMATION OF MARGIN FOR COMMERCIAL VS. GOVERNMENT PAY

The RAND data for Med Center Health estimates commercial insurance reimbursements at 246% of Medicare Rates

Estimating Commercial Margin

(Calculated through counterfactual of assuming all CMS/Government pay was paying at commercial rates)

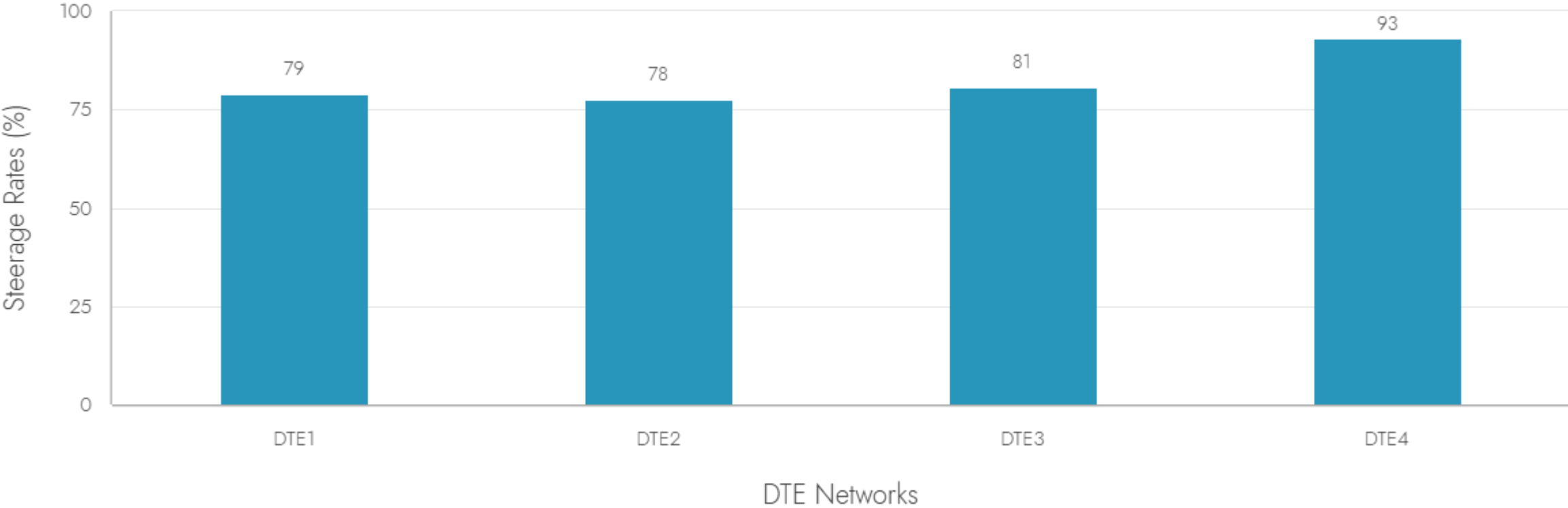
Commercial Revenue	\$403,658K
Gov-pay revenue at 246% current rate	\$650,936K
Total Revenue	\$1,054,595K
Total Costs	\$614,521K
Net Income	\$440,074K
	42%

Estimating Gov-Pay Margin

(Calculated through counterfactual of assuming all commercial pay was paying at Medicare rate)

Commercial Revenue	\$163,909K
Divided by 246%	
Gov-pay revenue	\$264,318K
Total Revenue	\$428,227K
Total Costs	\$614,521K
Net Income	\$ -186,293K
	-44%

STEERAGE RATES FOR CURRENT DTE NETWORKS



The average Tier 1 Steerage is 83%.

PROJECTED PLAN GROWTH

Year	Employees	Members	Percent of Members Choosing Med Center Health for Inpatient Care	Percent of Members Choosing Med Center Health for Outpatient Care
1	100	200	77%	70%
2	200	400	77%	70%
3	400	700	77%	73%
4	600	1,100	77%	74%
5	800	1,400	77%	74%
6	1,100	2,000	77%	74%
7	1,500	2,700	77%	74%
8	1,900	3,400	77%	74%
9	2,400	4,300	77%	74%
10	2,900	5,200	77%	75%
11	3,400	6,100	77%	75%
12	4,000	7,200	77%	75%
13	4,400	7,900	77%	75%
14	4,900	8,800	77%	75%
15	5,200	9,400	77%	75%
16	5,500	9,900	77%	75%
17	5,800	10,400	77%	75%
18	6,000	10,800	77%	75%
19	6,100	11,000	77%	75%
20	6,200	11,200	77%	76%

KBA currently estimates that Med Center Health has 70% inpatient market share, and 50% outpatient market compared to hospitals/clinics in the Med Center Health service area (see slide 8).

Based on previous DTE experience, KBA expects a significant bump in patient steerage to Med Center Health.

Additional steerage will occur from additional steerage patient benefits and the patient convenience card.

High estimates start nearly 10% higher than target with low estimates at 10% lower than target

YEAR 4 PROJECTED REVENUE AND PROFIT

Pre-DTE Cohort of 1,000 Employees	Inpatient	Outpatient	Professional	Total
Revenue†	\$ 1,928,560	\$ 2,504,403	\$ 1,169,535	\$ 5,602,497
Net Income‡	\$ 647,996.1	\$ 841,479.3	\$ 392,963.7	\$ 1,882,439

With DTE	Inpatient	Outpatient	Professional	Total
Revenue°	\$ 2,122,614	\$ 3,693,449	\$ 1,724,809	\$ 7,540,872
Net Income*	\$ 713,198.2	\$ 1,240,999	\$ 579,536	\$ 2,533,733

† Assuming proportional revenues from market shares of 70% (inpatient) and 50% (outpatient)

‡ Conservatively assuming a commercial margin of 33.6% (80% of the estimated commercial margin)

° Assuming 77% inpatient and 74% market steerage based on KBA narrow network DTE experience (see previous slide)

* Conservatively assuming a commercial margin of 33.6%, with a 10 percentage-point reduction to reflect competitive-edge rates and a 10 percentage-point increase due to lower billing costs and higher patient responsibility collections because of the member convenience card.

Total Increase in Net Income	\$ 651,294.1
Network Access Fee (Revenue) of \$1 PMPM	\$ 12,000
Total Profit Growth	\$ 663,294.1
Minus Implementation Fee (One-Time)	-\$ 0
Minus Sales and Marketing (Annual – Ongoing)	-\$ 180,000
Total	\$ 483,294.1

Target Estimate Assuming 50%+ Within-Plan Steerage and a Commercial Margin of 33.6%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Inpatient Revenue	\$964.3	\$1,928.6	\$9,642.8	\$48,214.0
Pre-DTE Group Inpatient Margin	\$324.0	\$648.0	\$3,240.0	\$16,199.9
DTE Group Inpatient Revenue	\$1,062.8	\$2,125.7	\$10,628.5	\$53,142.3
DTE Group Inpatient Margin	\$357.1	\$714.2	\$3,571.2	\$17,855.8
DTE Group Incremental Inpatient Revenue	\$98.6	\$197.1	\$985.7	\$4,928.3
DTE Group Incremental Inpatient Margin	\$33.1	\$66.2	\$331.2	\$1,655.9
Pre-DTE Group Outpatient Revenue	\$1,252.2	\$2,504.4	\$12,522.0	\$62,610.1
Pre-DTE Group Outpatient Margin	\$420.7	\$841.5	\$4,207.4	\$21,037.0
DTE Group Outpatient Revenue	\$1,759.6	\$3,519.1	\$17,595.6	\$87,977.9
DTE Group Outpatient Margin	\$591.2	\$1,182.4	\$5,912.1	\$29,560.6
DTE Group Incremental Outpatient Revenue	\$507.4	\$1,014.7	\$5,073.6	\$25,367.9
DTE Group Incremental Outpatient Margin	\$170.5	\$340.9	\$1,704.7	\$8,523.6
Pre-DTE Group Professional Revenue	\$584.8	\$1,169.5	\$5,847.7	\$29,238.4
Pre-DTE Group Professional Margin	\$196.5	\$393.0	\$1,964.8	\$9,824.1
DTE Group Professional Revenue	\$821.7	\$1,643.4	\$8,217.0	\$41,084.9
DTE Group Professional Margin	\$276.1	\$552.2	\$2,760.9	\$13,804.5
DTE Group Incremental Professional Revenue	\$236.9	\$473.9	\$2,369.3	\$11,846.6
DTE Group Incremental Professional Margin	\$79.6	\$159.2	\$796.1	\$3,980.5
Net Margin Less DTE Fees	\$-40.8	\$248.4	\$2,562.0	\$14,130.0

Target Estimate Assuming 50%+ Within-Plan Steerage and a Commercial Margin of 33.6%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Total Revenue	\$2,801.2	\$5,602.5	\$28,012.5	\$140,062.4
Pre-DTE Group Total Margin	\$941.2	\$1,882.4	\$9,412.2	\$47,061.0
DTE Group Total Revenue	\$3,644.1	\$7,288.2	\$36,441.0	\$182,205.2
DTE Group Incremental Total Revenue	\$842.9	\$1,685.7	\$8,428.5	\$42,142.7
Incremental Total Revenue by Member	\$1.7	\$1.7	\$1.7	\$1.7
DTE Group Total Margin	\$1,224.4	\$2,448.8	\$12,244.2	\$61,220.9
DTE Group Incremental Total Margin	\$283.2	\$566.4	\$2,832.0	\$14,160.0
Wellness Share	\$6.0	\$12.0	\$60.0	\$300.0
Implementation Fee (One-Time)	\$150.0	\$150.0	\$150.0	\$150.0
Sales and Marketing Annual Fee	\$180.0	\$180.0	\$180.0	\$180.0
Net Margin Less DTE Fees	\$-40.8	\$248.4	\$2,562.0	\$14,130.0

High Estimate Assuming 60%+ Within-Plan Steerage and a Commercial Margin of 38.6%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Total Revenue	\$2,801.2	\$5,602.5	\$28,012.5	\$140,062.4
Pre-DTE Group Total Margin	\$941.2	\$1,882.4	\$9,412.2	\$47,061.0
DTE Group Total Revenue	\$4,117.2	\$8,234.4	\$41,171.9	\$205,859.4
DTE Group Incremental Total Revenue	\$1,315.9	\$2,631.9	\$13,159.4	\$65,796.9
DTE Group Total Margin	\$1,486.3	\$2,972.6	\$14,863.0	\$74,315.2
DTE Group Incremental Total Margin	\$545.1	\$1,090.2	\$5,450.9	\$27,254.3
Wellness Share	\$6.0	\$12.0	\$60.0	\$300.0
Implementation Fee (One-Time)	\$150.0	\$150.0	\$150.0	\$150.0
Sales and Marketing Annual Fee	\$180.0	\$180.0	\$180.0	\$180.0
Net Margin Less DTE Fees	\$221.1	\$772.2	\$5,180.9	\$27,224.3

Low Estimate Assuming 40%+ Within-Plan Steerage and a Commercial Margin of 28.6%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Total Revenue	\$2,801.2	\$5,602.5	\$28,012.5	\$140,062.4
Pre-DTE Group Total Margin	\$941.2	\$1,882.4	\$9,412.2	\$47,061.0
DTE Group Total Revenue	\$3,178.9	\$6,357.9	\$31,789.4	\$158,946.8
DTE Group Incremental Total Revenue	\$377.7	\$755.4	\$3,776.9	\$18,884.4
DTE Group Total Margin	\$988.6	\$1,977.3	\$9,886.5	\$49,432.5
DTE Group Incremental Total Margin	\$47.4	\$94.9	\$474.3	\$2,371.5
Wellness Share	\$6.0	\$12.0	\$60.0	\$300.0
Implementation Fee (One-Time)	\$150.0	\$150.0	\$150.0	\$150.0
Sales and Marketing Annual Fee	\$180.0	\$180.0	\$180.0	\$180.0
Net Margin Less DTE Fees	\$-276.6	\$-223.1	\$204.3	\$2,341.5

Med Center Health — Contracted Inpatient Rates by Payer

Commercial plans, case rate methodology | All values as multiple of Medicare

MED CENTER HEALTH			
Payer / Plan	Sample Size	Mean × Medicare	Median × Medicare
Aetna — All Plans	739	3.16x	3.17x
Cigna — All Plans	739	3.16x	3.17x
United Healthcare — All Plans	739	3.16x	3.17x
Humana — All Plans	743	3.14x	3.15x
Anthem — Blue Traditional / Access / Preferred	722	3.08x	3.09x
Anthem — Pathway HMO	722	2.83x	2.84x
Anthem — Pathway HPN	722	2.62x	2.63x
Center Care Select	733	2.61x	2.61x
Anthem — Pathway Transitions HMO	722	2.16x	2.16x

Key Insight: Commercial inpatient rates cluster at 3.08x–3.16x Medicare for major payers, demonstrating consistent and strong pricing across MCH's commercial payer portfolio.

Norton Healthcare — Contracted Inpatient Rates by Payer

Commercial plans, case rate methodology | All values as multiple of Medicare

NORTON HEALTHCARE

Payer / Plan	Sample Size	Mean × Medicare	Median × Medicare
Aetna Adult — Commercial	746	2.34x	2.35x
United Healthcare Adult — Commercial	752	1.91x	1.91x
Cigna — Commercial	753	1.66x	1.65x
Cigna Adult — Commercial	746	1.66x	1.65x
Aetna Alternative — Commercial	746	1.64x	1.64x
Anthem Adult — HMO/PPO/Traditional	710	1.56x	1.56x

Commercial Inpatient Rate Differential

MCH vs. Norton — Mean × Medicare by Common Payer

Payer	MCH Mean	Norton Mean	Absolute Diff.	MCH Premium
Aetna	3.16x	2.34x	+0.82x	+35%
United Healthcare	3.16x	1.91x	+1.25x	+66%
Cigna	3.16x	1.66x	+1.50x	+90%
Anthem (Primary)	3.08x	1.56x	+1.52x	+97%
Humana	3.14x	N/A	—	—

Lowest Premium

Aetna: MCH 35% above Norton — still a significant advantage.

Highest Premium

Anthem: MCH nearly 2× Norton's rate for primary commercial plans.

Negotiation Note

Expect Norton benchmarking pressure; prepare rationale for differential.

Note: Norton's government managed care rates are not shown here; commercial comparison only.

Contracted Outpatient (HCPCS) Rates

Mean × Medicare — selected payers, case rate methodology

MED CENTER HEALTH		
Payer / Plan	Mean ×	Median ×
Humana — All Plans	2.81x	2.33x
Self-Pay	2.65x	1.87x
Anthem Blue Traditional	2.46x	2.02x
Anthem Pathway HMO	2.29x	1.86x
Center Care	2.20x	1.50x
Cigna — All Plans	2.18x	1.52x
Aetna — All Plans	2.15x	1.50x
United Healthcare	2.13x	1.48x

NORTON HEALTHCARE		
Payer / Plan	Mean ×	Median ×
UHC Pediatric	4.28x	4.12x
Anthem Children's Pathway	4.26x	4.05x
Correct Care (OB & GYN)	4.21x	3.95x
Anthem Pediatric	4.08x	3.81x
Multiplan	4.04x	3.73x
UHC Adult	2.55x	2.05x
SIHO	2.40x	1.89x
Anthem Adult	2.20x	1.60x

↑ Norton's elevated outpatient averages are driven by pediatric and specialty contracts unavailable at MCH. Comparing adult commercial plans, MCH's Humana (2.81x mean) outperforms Norton's comparable adult tiers.

Government & Managed Government Payer Rates

Medicare Advantage and Managed Medicaid — Inpatient DRG | Case rate methodology

MED CENTER HEALTH

Payer / Plan	Mean ×	Median ×
Medicare Advantage (Aetna)	1.24x	1.24x
Medicare Advantage (Anthem)	1.24x	1.24x
Medicare Advantage (Humana)	1.24x	1.24x
Medicare Advantage (UHC)	1.24x	1.24x
Managed Medicaid (Anthem)	1.14x	1.14x
Managed Medicaid (Humana)	1.14x	1.14x
Managed Medicaid (Aetna)	1.11x	1.11x

1.11x–1.24x

MCH Gov. Range

Medicare Adv. & Medicaid MA

NORTON HEALTHCARE

Payer / Plan	Mean ×	Median ×
Wellcare — Medicare Advantage	1.37x	1.37x
UHC — Medicare Advantage	1.37x	1.36x
Aetna — Medicare Advantage	1.36x	1.36x
Anthem Senior MA	1.36x	1.36x
Passport — Managed Medicaid	1.39x	1.39x
Humana CareSource — Medicaid	1.37x	1.37x
Anthem of KY — Medicaid	1.36x	1.36x
Aetna Better Health KY	1.32x	1.32x

1.32x–1.39x

Norton Gov. Range

Medicare Adv. & Medicaid MA



IMPLEMENTATION AND OPERATIONS

Claims Processing Excellence

- Delivering a positive member and provider experience starts with accurate, efficient, and transparent claims processing
- **Key Benefit Administrators leverages:**
 - Proven technology
 - Streamlined workflows
 - A team dedicated to first pass accuracy
- **We Consistently:**
 - Exceed industry standard turnaround times and quality metrics
 - Reduce administrative burdens
 - Maintain high member and provider satisfaction
- **Strategic Partnerships:**
 - We partner with **Smart Data Solutions** for clearinghouse services to empower providers with electronic claims submission and transparency tools.
 - Currently 82% of our claim volume is received electronically.
 - Ongoing efforts to increase electronic submissions:
 - Analyze paper and non-electronic claims
 - Proactively engage providers for electronic claim setup
 - We also partner with top tier vendors to provide cost containment solutions to include:
 - Out of network discounts
 - High dollar bill reviews
 - Specialized networks for high-cost conditions such as transplants and dialysis.



Claims Statistics

Claims	Industry Standard	Corporate Goal	Self-Funded Medical Achieved*
Turn Around Time	10 calendar days	7 calendar days	4.25 calendar days
% Paid in 10 days	80%	85%	86.7%
Financial Quality	99%	99%	99.6%
Payment Accuracy	NA	95%	97.4%

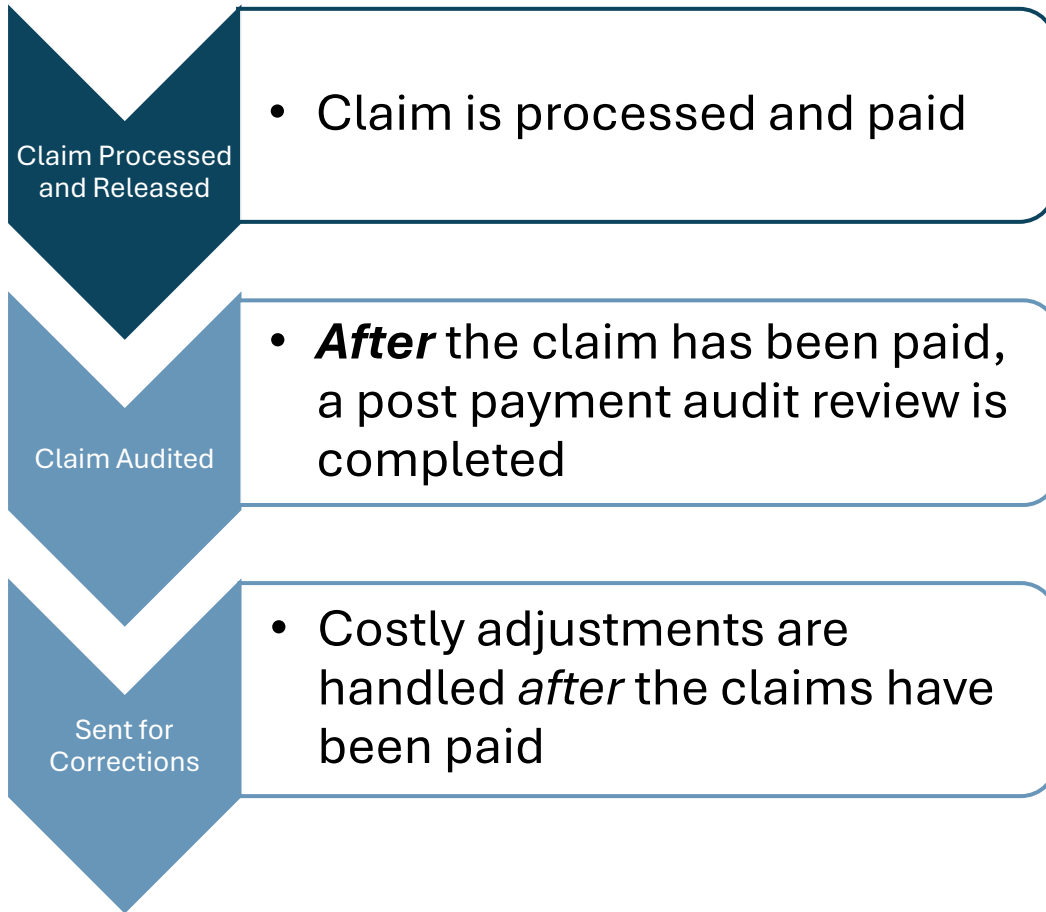
* Data achieved is for Jan-Jun 2025 calendar year

- Groups are provided with a dedicated specialist to handle escalated issues and concerns promptly.
- We offer comprehensive, interactive training for claims and customer service staff to prepare them for their roles, using a collaborative LMS system to ensure consistency and accuracy in KBA processes.



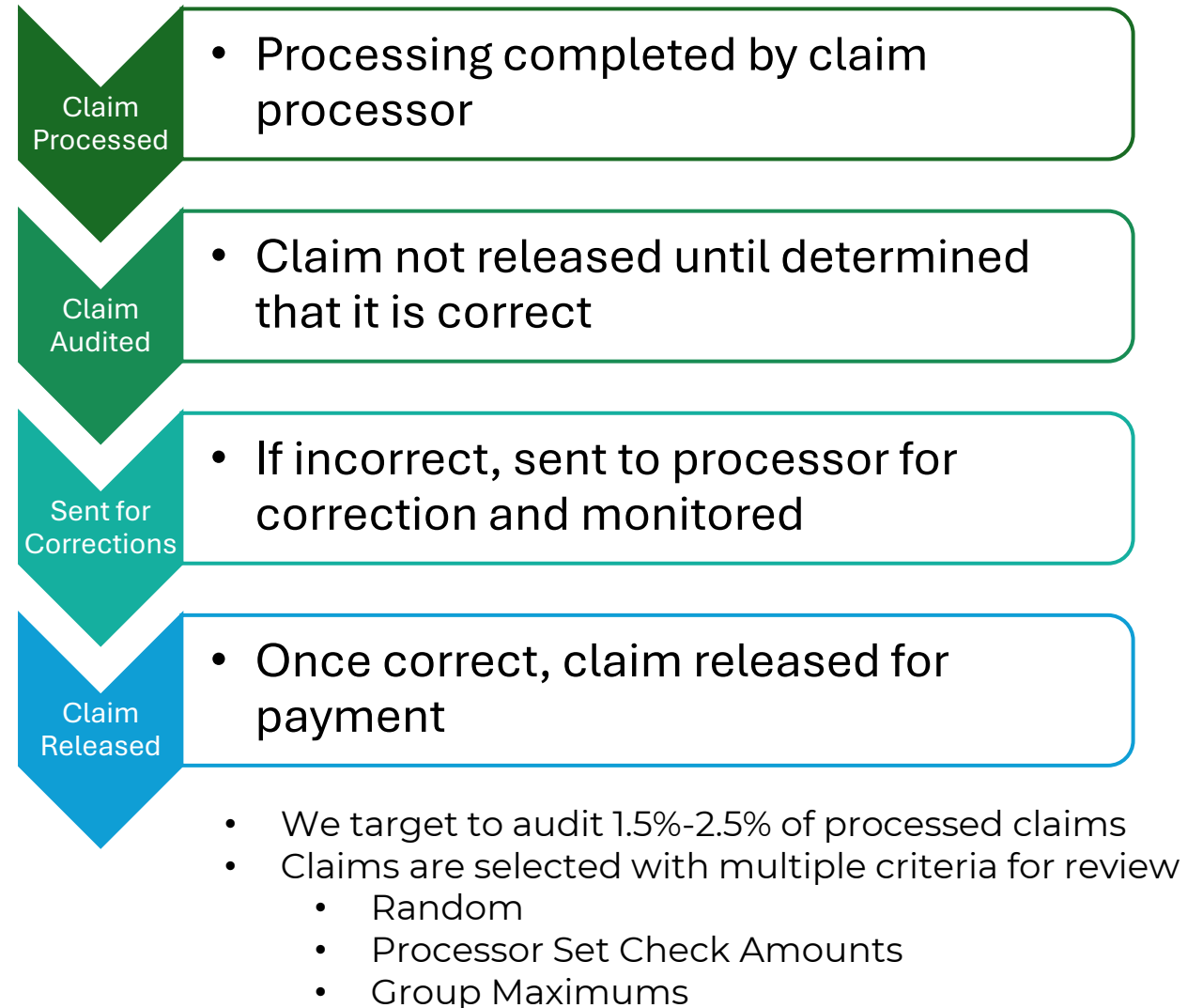
Industry Claim Audits

Post Payment Reviews



KBA Claim Audits

Pre Payment Reviews



KBA 2023-2024 Total Quality Management Statistics

300+
TOTAL QUALITY MEASURES

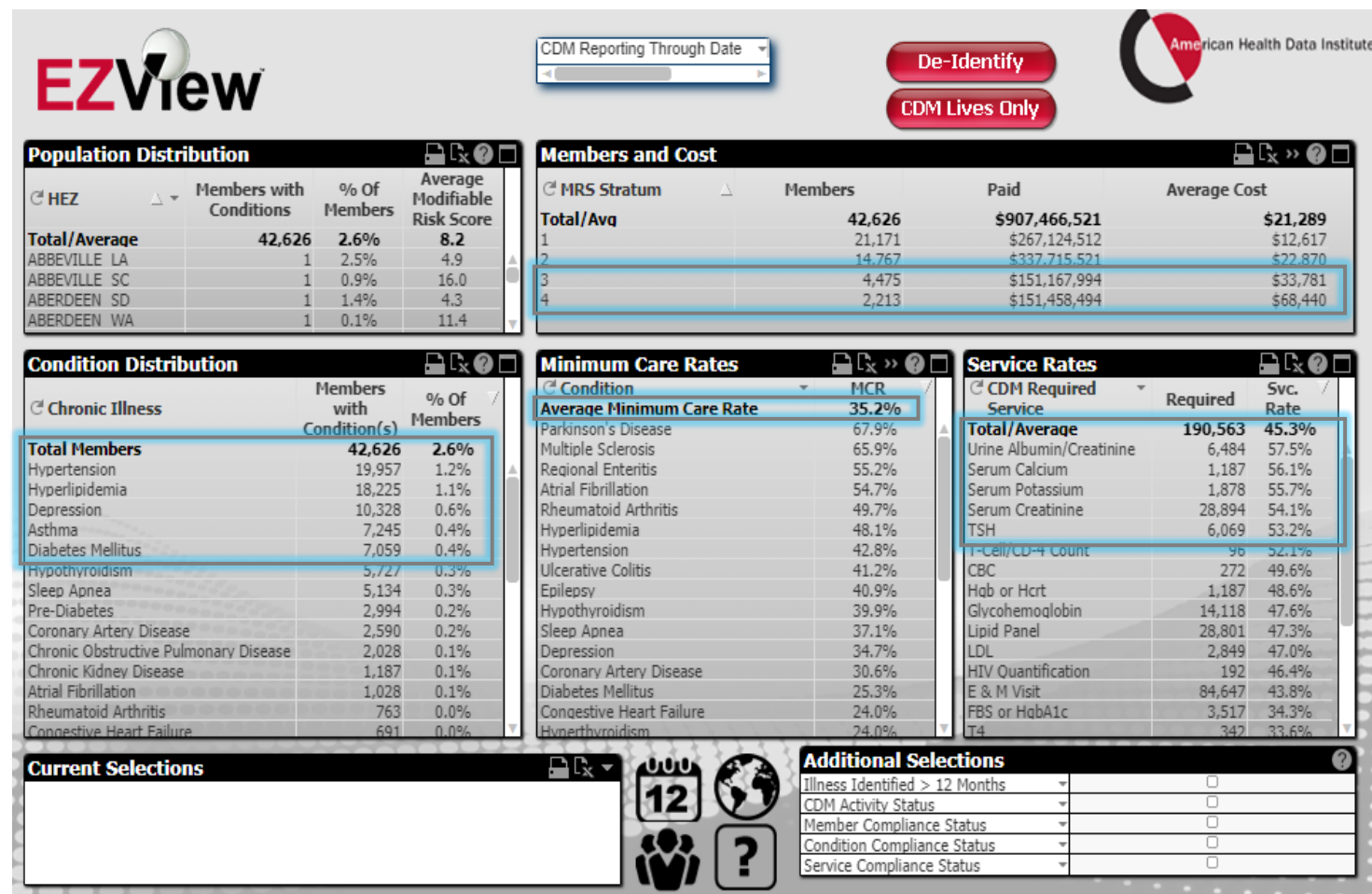
50,000+
UNIQUE AUDITS ANNUALLY

\$200M+
PREMIUM & CLAIMS REVIEWED ANNUALLY

New Business / Renewals					Enrollment Center				
New Case Accuracy	2023 KBA 99.86%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 99.75%	Soft Skills Audit	2023 KBA 99.43%	2024 KBA 99.22%	INDUSTRY N/A	GOAL 98.00%
Renewal Case Accuracy	2023 KBA 99.96%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 99.75%	Procedural Skills Audit	2023 KBA 98.26%	2024 KBA 98.23%	INDUSTRY N/A	GOAL 97.00%
Fulfillment Materials Audit	2023 KBA 99.99%	2024 KBA 99.98%	INDUSTRY N/A	GOAL 100%					
Billing & Eligibility Quality					Claims Quality				
Eligibility Accuracy	2023 KBA 100.00%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 99.75%	Turn Around Time (Days)	2023 KBA 3.79	2024 KBA 6	INDUSTRY 10	GOAL 7
Plans/Rates/Coverage Audits	2023 KBA 100.00%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 99.75%	% Paid within 10 Bus Days	2023 KBA 88.90%	2024 KBA 87.34%	INDUSTRY 80.00%	GOAL 85.00%
Cash Receipts Logging	2023 KBA 99.97%	2024 KBA 99.78%	INDUSTRY N/A	GOAL 98.00%	Financial Accuracy	2023 KBA 99.30%	2024 KBA 99.44%	INDUSTRY 99.00%	GOAL 99.00%
Cash Posting Accuracy	2023 KBA 99.96%	2024 KBA 99.78%	INDUSTRY N/A	GOAL 97.00%	Procedural Accuracy	2023 KBA 99.00%	2024 KBA 99.16%	INDUSTRY N/A	GOAL 95.00%
					% of Claims Audited	2023 KBA 3.77%	2024 KBA 3.80%	INDUSTRY 1%	GOAL 3%
Customer Experience Quality									
Average Speed to Answer	2023 KBA 22 sec	2024 KBA 44 sec	INDUSTRY <45s	GOAL <30s	% Answered w/in 45 Sec	2023 KBA 84.90%	2024 KBA 75.00%	INDUSTRY 80-60%	GOAL 80-30%
Soft Skills Audit	2023 KBA 99.50%	2024 KBA 99.54%	INDUSTRY N/A	GOAL 98.00%	Procedural Audit	2023 KBA 99.22%	2024 KBA 99.10%	INDUSTRY N/A	GOAL 97.00%
Email Correspondence Audit	2023 KBA 100.00%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 98.00%	Web Chat Corresp. Audit	2023 KBA 99.64%	2024 KBA 99.50%	INDUSTRY N/A	GOAL 98.00%
Abandonment Rate	2023 KBA 1.00%	2024 KBA 1.78%	INDUSTRY <5%	GOAL <1%					

TRANSPARENT ANALYTICS ACCESS

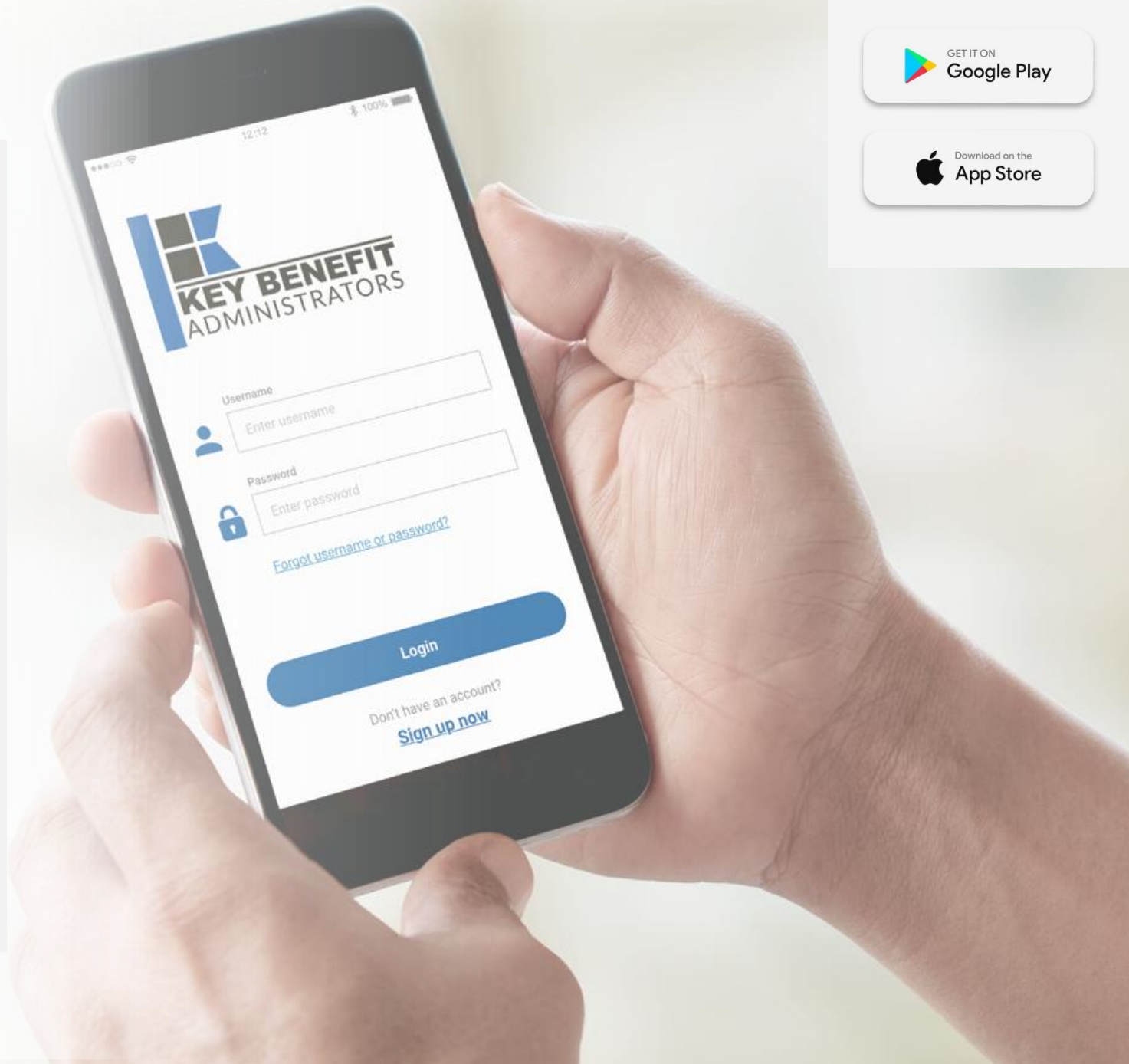
- EZView analytics platform provides instant access to critical analytics insights.
- Filter to a population by data source, group, plan, or geographic location.
- Identifications of chronic conditions, patient acuity, compliance with nurse coaching, regimens of care, steerage rates or claims costs.



Web Portal and Mobile Application

Gives you accessibility to your benefit plan information, claims data, eligibility details and much more!

- Biometric Login for ease of access (Fingerprint & Facial Recognition)
- Simplified Accumulator View
- Recent Claims Summary and Full Detail
- Benefit Information
- View/Request an ID Card (Display - Print – Fax – Email)
- SSO directly to Vendor Solutions
- Rx Price Comparison and Pharmacy Discount Programs (if applicable)
- Express Requests to Customer Support
- Push Notifications (Mobile App)



GET IT ON
Google Play

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App Store

Measuring Success: Customer Experience

Key Performance Indicators

Metric	2025 YTD	Industry Benchmark	How We Compare	
Abandonment Rate	1.04%	3–5% (acceptable), <2% (best-in-class)	Excellent 🏆	✓ Industry-leading responsiveness 1.04% abandonment and 17 s average speed of answer—minimal dropped calls, maximum access. ✓ Outstanding quality & access 90% of calls answered within 30s (vs. 80% target) and 99%+ audit scores across voice, email & chat. ✓ Balanced efficiency & depth Handle time reflects the detailed, compassionate TPA support your members need.
Average Speed of Answer (ASA)	17 seconds	<28 seconds (typical target); ~3 min in healthcare BPO	Excellent 🏆	
SLA (80% ≤ 30 sec)	90.00%	80% in 20–30 seconds (standard)	Above target ✅	
Average Handle Time	7 minutes 47 seconds	4–6 minutes typical; 7–8+ mins for complex TPA calls	Higher than industry, but expected	
After Call Work (ACW)	49 seconds	30–60 seconds typical	Within range 🟢	
Quality Audits (Email, Chat, Soft Skills, Procedural)	99%+	95–97% considered strong	Outstanding 100	

KBA DEDICATED TEAM ROLES

Our expert team is committed to delivering tailored solutions and exceptional service, ensuring consistent support and strategic guidance for every client.

Client Vice President (CVP)

Average Tenure: 15 Years

A dedicated individual delivering solutions tailored to client needs, driven by their unique data. The KBA CVP is deeply knowledgeable about client demographics, goals, and plans, as well as the health status of their membership. Through unparalleled analytics, the CVP illuminates the intelligence of the data, coordinates and conducts plan performance reviews, and provides meaningful recommendations. The CVP can also assist with employee enrollment meetings, presentations, and announcement materials, staying informed on client issues and initiatives, and tracking all account-related information.

Benefit Consultant (BC)

Average Tenure: 5 Years

Delivers strategic consultation to the client in conjunction with the CVP, with a primary focus on reporting and strategic insights. During renewal season, the BC assists in preparing comprehensive renewal proposals that include detailed service offerings, clear fee structures, and defined timelines. They also coordinate and conduct employee enrollment meetings as needed, ensuring a seamless and informed process for all stakeholders.

Account Representative (AR)

Average Tenure: 3 Years

Manages the day-to-day needs of the account, ensuring consistent communications, timely follow-up services, and continuous account monitoring. The AR oversees new case implementation and renewal processes alongside the KBA operations team. Responsibilities include the distribution of monthly reports, facilitating plan design changes, and obtaining signatures for plan documents and amendments. Additionally, they assist with funding management, including ACH transfer of funds and various other inquiries to ensure smooth financial operations.



ADDITIONAL KBA TEAM MEMBERS

On-Site KBA Support

An on-site resource for members to ask questions regarding benefits, eligibility, or claims. This role also provides essential HR support for support for plan administration, ensuring smooth operations and direct assistance.

Economist/Analyst

George Avery, PhD, MPA: Dr. Avery holds a doctorate in Health Services Research, Policy, and Administration from the University of Minnesota. With extensive academic experience as a professor at Purdue University and the University of Minnesota Duluth, he has contributed significantly to his field. His impressive publication record includes six book chapters and over 20 peer-reviewed journal articles.

Dr. Avery's expertise is highly sought after, evidenced by his advisory roles for prestigious organizations such as the Indiana Office of Rural Health, the Centers for Disease Control and Prevention, the Department of Defense, and the United States Environmental Protection Agency. He brings over 30 years of invaluable experience in public health, health economics, and econometric analysis to the team.



DTE Implementation Team

The DTE Implementation Team is responsible for the overall management of the DTE project plan for the DTE Network and KBA Administrative Operations.

We are always excited about new partnerships and look forward to developing a solid foundation to build upon.



Christine Vinesett

Director, Implementation and Network

I joined Key Benefit Administrators in 2000 and have been in the Health Insurance Industry for 20+ years. My focus has been Leadership in Operations, Carrier and Provider Network Relations and Implementation Management.

Office: (803) 323-7974 | Email: Christine.Vinesett@keybenefit.com



Julie Caskey

Network Operations Lead

Julie Caskey joined Key Benefit Administrators (KBA) in 2021. Julie brings an extensive background with 30+ years in the Health Insurance Industry. Julie's focus has been in Provider Network and Relationship Management.

Office: 317-712-4003 | Email: Julie.Caskey@keybenefit.com



Dejanetta (Deja)
Pettaway

DTE Implementation and Relationship Coordinator

Deja joined Key Benefit Administrators in 2023 and has been in the Health Insurance Industry for 10 years. Deja's focus has been in Provider Network and Relationship Management.

Office: 803-396-4836 | Email: Dejanetta.Pettaway@keybenefit.com



Darlene Grindstaff

DTE Implementation and Relationship Coordinator

Darlene Grindstaff joined Key Benefit Administrators in 2014 and has been in the Health Insurance Industry for 11 years. Darlene's focus has been in Implementation, Provider Network, and Relationship Management.

Office: 803-396-4784 | Email: Darlene.Grindstaff@keybenefit.com

DTE IMPLEMENTATION OVERVIEW

Phase 1	Phase 2		Phase 3	
DTE Network Setup	DTE Marketing & Website Setup	DTE Provider File Setup	Group Sales & Plan Design Setup	Group Administration Setup
☐ Legal Contracts and Agreements	☐ DTE Name Established	☐ Provider data and file analysis	☐ Group Sales and Marketing Strategy	☐ Proposal system setup
☐ Security and Compliance review	☐ DTE logo and messaging	☐ Provider file requirements	☐ Group Sales collateral	☐ Group Implementation and Renewal Collateral
☐ Secure email connection - Transport Layer Security (TLS)	☐ DTE Marketing Collateral	☐ Provider file transfer	☐ Group plan design and benefit structure	☐ Administration system programming (DTE product, plan design, billing, distribution, extract files)
☐ Secure file and reporting connection - Secure File Transfer Protocol (SFTP)	☐ DTE Website URL (domain)	☐ Provider file programming and website testing	☐ PBM plans and benefits review	☐ Claims adjudication system programming
☐ Network review of providers, specialties and sub-specialties	☐ DTE Website design and content creation	☐ Provider maintenance file established	☐ PBM structure analysis (systems, data import, data export, billing and invoicing)	☐ Fulfillment programming
☐ Network contracting and pricing analysis	☐ DTE Website provider portal setup	☐ DTE Website review for launch	☐ PBM file structure programming and testing (claims, accumulators, elig, invoice feed)	☐ Customer Service programming
☐ Network fee schedule programming and testing			☐ Reinsurance Carrier education on DTE	☐ Complete End to End Testing
Network launch	Website launch		Group Sales launch	

Group Implementation and Client Relations Team

Group implementations are assigned members from the Implementation and Client Relations Team. KBA is committed to delivering a “WOW” level of customer service and our team will ensure a smooth and timely transition to KBA.

It is our priority to execute a successful implementation.

Group Implementation Leadership

- Implementation Director
 - Christine Vinesett
 - Email: Christine.Vinesett@keybenefit.com | Phone: (803) 323-7974
- Implementation Manager
 - Rachel Morris
 - Email: Rachel.morris@keybenefit.com | Phone: (803) 396-4775
- Implementation Lead
 - Ashley Gowen
 - Email: Ashley.gowen@keybenefit.com | Phone: (317) 284-7863
- ❖ Implementation Coordinator's will be assigned at kick-off.

Client Relations Leadership

- Senior Vice President, Client Relations
 - Nicole Oliver
 - Email: Nicole.oliver@keybenefit.com | Phone: (803) 396-4720
- ❖ Client Relations VP and Account Representative will be assigned at kick-off.

GROUP IMPLEMENTATION OVERVIEW

Phase 1	Phase 2	Phase 3	Phase 4
Implementation Begins	Group Details - Open Enrollment, Eligibility Setup, ID Card, Banking, and Vendors	Finalize Setups - Open Enrollment, Eligibility, ID Card, Banking and Vendors	Confirmations - Fulfillment, Post Implementation Planning, GO LIVE!
☐ Identify key group contacts and their Roles and Responsibilities.	☐ Confirm Open Enrollment dates.	☐ Introduce Billing and Customer Service Teams.	☐ Confirm eligibility loaded.
☐ Review contracts and electronic signature process.	☐ Discuss materials needed for Open Enrollment and expected delivery.	☐ Process Open Enrollment eligibility.	☐ Confirm fulfillment mail date.
☐ Confirm Signor information.	☐ Discuss COBRA Administration. (if applicable)	☐ Finalize COBRA Administration setup. (if applicable)	☐ Confirm COBRA letter and packet mail dates. (if applicable)
☐ Establish meeting schedule, frequency and set target dates.	☐ Discuss Customer Service setup.	☐ Finalize EZ Benefits setup.	☐ Confirm all group setups are complete.
☐ Begin KBA Client Profile.	☐ Review EDI Setup and initial eligibility file submission.	☐ Finalize ID Card setup.	☐ Confirm plan document has been signed.
☐ Request prior plan document.	☐ Schedule EDI meeting. (if applicable)	☐ Finalize banking setups.	☐ Deliver 1st Months Invoice and finalize.
☐ Review plan document, benefits and discuss changes.	☐ Review EZ Benefits platform.	☐ Finalize claims and EOB setups.	☐ Deliver final reinsurance contract.
☐ Request claim file and accumulator data.	☐ Discuss ID Card setup, approval process and delivery expectations.	☐ Finalize PBM Implementation.	☐ Discuss reporting needs for plan year.
☐ Review Fee Schedule, Implementation Fee.	☐ Discuss Banking and Funding setup for monthly premiums and claims.	☐ Finalize Plan Document.	☐ Complete full transition to Client Relationship Team for administration.
☐ Discuss Notice of Termination to prior carrier and obtain copy.	☐ Discuss frequency of check runs, large dollar claims review, exception process and EOBs.	☐ Review status of EDI setup. (if applicable)	
	☐ Discuss monthly premium billing preferences.		
	☐ Begin PBM Implementation and schedule meetings.		
	☐ Discuss vendor setup process.		
	☐ Review special programs. (if		

COLLABORATION OF STAKEHOLDERS



Key Benefit Administrators

- Full TPA Administration
- Stop-Loss Administration
- True Transparency & Analytics
- Population Health Management
- Utilization & Case Management
- Sales & Marketing
- Transparent PBM
- Website & Mobile App
- Customizable options & plans



Med Center Health's DTE

- Incremental Revenues and Margin
- Branding & Messaging
- Benefit Plan Integration
- Network Steerage
- Direct Relationship with Self-Funded Employers
- Referral Authorization
- Plan Transparency
- Direct Access to Members



Employer

- Reduced Spend
- "At The Table" with Med Center Health
- Improved Analytics
- Complete Transparency
- Lower Stop-Loss
- Benefit Plan Stabilization
- Healthier Population



Employee

- Reduced Out of Pocket
- Improved Health
- Better Coordination & Navigation of Care
- Direct Access to the Best Medical Providers

ROLES & RESPONSIBILITIES

SERVICES & ADMINISTRATIVE SYSTEMS PROVIDED BY:	
Med Center Health	KBA Operations
▪ Tier 1 Provider Network ▪ Provider Recruitment – If Necessary ▪ Network Pricing & Contract Terms ▪ Referral Management Protocols & Approvals ▪ Medical Director Oversight ▪ Practice Improvement Program using KBA/AHDI Data with Assistance from KBA	▪ Network Evaluation – Actuarial Pricing ▪ PPO Wrap & Tertiary Transplant Network, if needed ▪ Claims Administration ▪ Plan Design Development, Testing, and Administration ▪ Employer/Member/Provider Customer Service ▪ Customized Member/Provider/Agent/HR Portal ▪ Billing, Enrollment (Electronic/Paper), and Premium Management ▪ Fulfilment – SPD & Customized ID Cards ▪ COBRA/HIPAA Administration ▪ Out of Network Negotiations ▪ Wrap PPO Repricing ▪ Nurse Coaching for Chronically Ill ▪ Nurse Triage to Physicians and Facilities ▪ Stop-Loss Underwriting, Pricing, and Placement ▪ Stop Loss Claims Administration ▪ Complex Case Management ▪ PBM Administration Interface ▪ Reporting • Utilization & Gaps in Care • Clinically Based Physician Profiling • Referrals • Chronic Disease Management ▪ Employer Health Risk Management Reports ▪ Ancillary Administration – Dental, Vision, Weekly Income, 5500, Life, etc. ▪ Health Savings Account (HSA), Flexible Spending (FSA), Health Reimbursement Account (HRA) Administration ▪ Sales & Marketing Plan • Strategy Development: Branding, Brochures, Web Site • Distribution System Development • Sales Management
KBA Consulting	
▪ Program oversight and integration across the enterprise ▪ Goal development, agreement and tracking ▪ Start-up assessments: market, network, operational and readiness ▪ Draft business plan with financial projections ▪ Facilitate program communication and engagement with Med Center Health departments including system c-suite, operations, HR, managed care, marketing, strategy and business development	

An aerial photograph of a city skyline under a blue sky with wispy clouds. A semi-transparent white rectangular box is centered over the middle of the image, containing the text 'NETWORK MANAGEMENT'. The city features various buildings, including a prominent brick building with a 'TIGER' sign on its roof, and a mix of greenery and urban infrastructure.

NETWORK MANAGEMENT

The Healthcare Provider Landscape Challenge

The Bowling Green Kentucky healthcare market presents a complex web of care providers, providers, specialty groups, and healthcare facilities that can overwhelm patients, employers, employers, and payers alike.

Market Complexity

- Hundreds of independent provider provider groups
- Multiple health system affiliations
- Varying quality and pricing structures
- Limited transparency in outcomes

The Challenge

Without strategic curation and management, this overwhelming number of number of care groups creates confusion, confusion, inefficiencies, and suboptimal suboptimal outcomes for all stakeholders. stakeholders.

The solution: A carefully managed, high-value network.



Our Goal in Partnering With Med Center Health



Build the Highest-Value Network

Create the area's premier healthcare network that delivers exceptional quality care with market-leading pricing, setting the gold standard for value-based healthcare delivery.



Enable Fastest-Possible Growth

Leverage our proven methodologies and established Med Center Health relationships to accelerate network expansion, ensuring rapid market penetration and comprehensive coverage across the greater Bowling Green KY area.



Vault Med Center Health to Competitive Advantage

Position Med Center Health as the anchor and leader of this transformative network, creating sustainable competitive advantages that drive market leadership and long-term success.

"Value is high-quality care at a great price – this principle guides every decision we make in network development and provider selection."



HEALTHCARE
PARTNERSHIP

Networks Must Be Carefully Tended

Proven Network Leadership

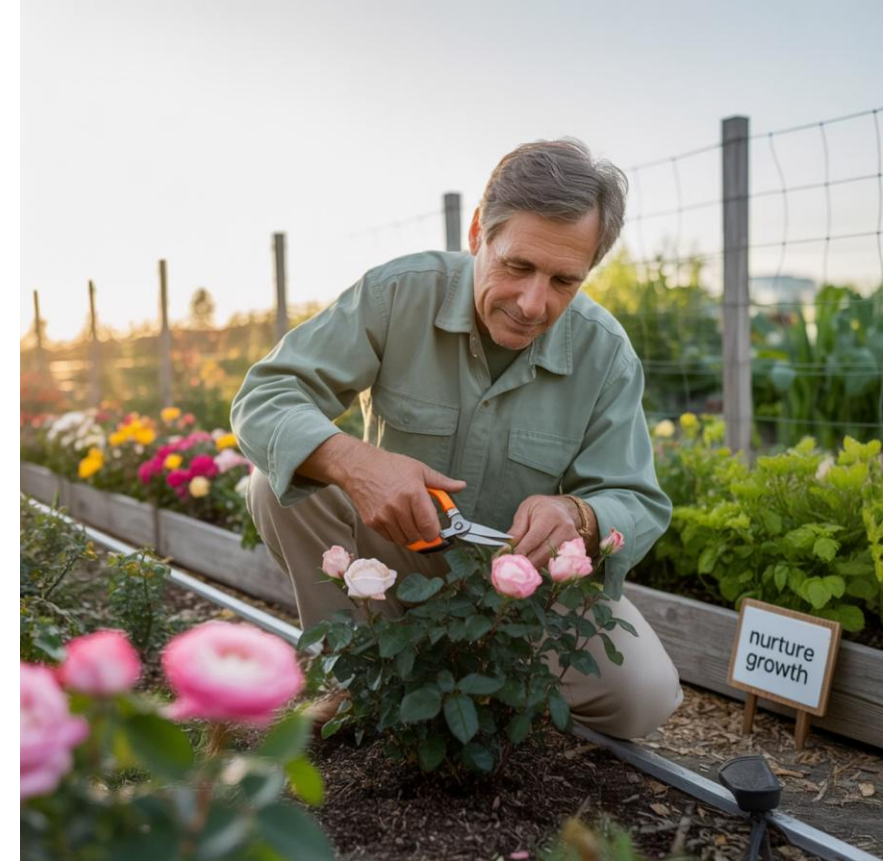
Our leadership team brings unparalleled depth of experience to healthcare network management management and strategic development.

Each team member has over 25 years of hands-on experience in healthcare network operations, operations, provider relations, strategic planning and contracting.

Our Management Approach

- **Yearly contracting cycles** – ensuring competitive rates and terms
- **Network integrity monitoring** – maintaining quality and access standards
- **Provider relations management** – fostering strong, productive partnerships
- **Performance analytics** – continuous improvement through data-driven insights insights

We manage all the tending always in cooperation with the sponsoring provider organization.



Vehicle for Approving Network Changes: Touch Point Meetings

Regular touchpoint meetings serve as the strategic command center for network development, ensuring every addition aligns with organizational goals and market needs.

01

Network Analysis

Comprehensive evaluation of current network gaps, utilization patterns, and performance metrics to identify strategic opportunities.

02

Stakeholder Feedback

Regular collection and analysis of feedback from employers, brokers, and members to ensure network meets real-world needs and expectations.

03

Option Review

Systematic evaluation of potential provider additions, considering clinical quality, geographic geographic coverage, and financial impact.

04

Strategic Recommendations

Data-driven recommendations presented to leadership for informed decision-making on network expansion and optimization.

05

Contracting Execution

Upon agreement, we approach and negotiate with target provider groups to secure favorable terms and seamless integration.



Who Needs to Be in the Touchpoints

i Critical Success Factor: Dedicated system leaders who can make real-time decisions about network inclusion and exclusion criteria.

Decision-Making Authority

Touchpoint meetings require participants to advise network changes, contract terms (where terms (where possible), and strategic direction.

Meeting Structure & Frequency

- **Weekly meetings** for the first several months during initial network build-out
- **Monthly meetings** once network reaches maturity
- **As-needed sessions** for urgent opportunities or issues

Required Participants

- Network leadership with contracting authority
- Clinical quality decision-makers
- Financial/pricing stakeholders
- Strategic planning representatives

The group collectively decides the meeting structure and governance approach that works best for their organization.

Contract Negotiations & Risk Management



Comprehensive Negotiation Management

We handle all contract language, terms, and subsequent subsequent reimbursement negotiations, ensuring optimal optimal terms while protecting your organization from legal from legal and financial risks.



Price-Fixing Protection

By managing all rate negotiations directly, we create a protective barrier that keeps Med Center Health safe from potential price-fixing concerns and antitrust issues.



Annual Renewal Excellence

All negotiations are renewed yearly, and our strong track record of delivering excellent reimbursements to providers enables smoother renewals and stronger relationships.

Our Negotiation Advantages

Our extensive experience across multiple markets gives us unique insights into fair market rates and industry best practices. We leverage practices. We leverage data analytics and market intelligence to secure favorable terms for both the network and participating providers. participating providers.





WE REMEMBER WHO IS GIVING THE CARE

A PARTNER YOU CAN TRUST



**OUR OBJECTIVES
ARE ALIGNED**



**WE KNOW THE
MARKET**



**COMPLETELY
TRANSPARENT**



**WE HAVE THE
SOPHISTICATION
AND
INFRASTRUCTURE
TO EFFECTIVELY
COMPETE**



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